

The Weather Within

A Bipolar Guide for Women —
Practical Strategies for Managing
Mood, Finding Stability, and Thriving

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For every woman who has spent her life feeling
different —
you are not broken.
You are wired differently, and that difference
has its own brilliance.

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Foreword: This Toolkit is for You

YOU'VE PROBABLY READ ENOUGH BOOKS THAT TELL YOU WHAT BIPOLAR DISORDER IS. MAYBE YOU'VE EVEN MEMORIZED THE DIAGNOSTIC CRITERIA AND STILL FELT LIKE NONE OF IT QUITE CAPTURED WHAT IT ACTUALLY FEELS LIKE TO LIVE INSIDE A BRAIN THAT OPERATES ON ITS OWN WEATHER SYSTEM.

This is not that kind of book.

This is a toolkit. A practical, grab-it-when-you-need-it collection of strategies, systems, and frameworks designed for women who are tired of feeling like their own mind is an unreliable narrator. Think of this book less like a textbook and more like a well-stocked emergency kit: flashlights where you can find them in the dark, instructions you don't have to decipher when you're already overwhelmed, and tools you've practiced using so they're muscle memory when you need them most.

Why This Book is Practical, Not Theoretical

There is no shortage of clinical literature on bipolar disorder. The research on mood stabilizers, neurochemistry, and genetic markers is deep and growing. And that knowledge matters. But there's a gap between understanding the science of bipolar and

actually living with it on a Tuesday afternoon in the middle of a work meeting when you feel the shift coming and you can't stop it.

This book lives in that gap.

Every chapter you're about to read was written with the same question in mind: What can you actually do with this information? Not "what does the research say?" (though research informs everything here), but "what do you do on Monday morning?" The tools in this book have been gathered from women who have been managing bipolar for decades—some diagnosed, some self-aware long before they had a name for it—and from clinicians who specialize in women's mental health. They have been tested in real kitchens, real bedrooms, real workplaces, real relationships.

Theory gives you a map of the territory. This book gives you a compass, a walking stick, and permission to stop and rest when you need to.

The Weather Pattern Metaphor

Throughout this book, you'll notice I talk about moods like weather. This isn't poetic license—it's a working metaphor that thousands of women have found genuinely useful for understanding and communicating what's happening inside them.

Here's why it works: Weather is something you cannot control, but you can prepare for, respond to, and ride out. You don't stand on the beach and yell at the tide to go back. When the forecast calls for snow, you adjust your plans, put on boots, drive slower. When a heatwave hits, you hydrate, seek shade, pace yourself.

Bipolar moods work the same way. Depression is not something you can "snap out of" any more than you can command a hurricane to disperse. Mania is not a character flaw you could resist if you just tried harder—it's a force of nature moving through your nervous system. Hypomania can feel like a perfect spring morning full of possibility, but it can also be the calm before a storm you don't see coming.

The weather metaphor gives you something crucial: distance. When you can say "a depressive front is moving in" instead of "I'm broken and falling apart," you create a sliver of space between you and the experience. That space is where all the tools in this book live.

Who This Book is For

If you were just diagnosed last week, you're in the right place. You're probably still reeling—from the name itself, from the implications, from the conversations you've already had or are dreading. You're wondering what this means for your life, your relationships, your career, your sense of who you are. This book will help you make sense of the diagnosis without letting it define you.

It will give you practical things to do starting today—not abstract advice about "acceptance" or "a journey," but concrete steps: what to track, what to ask your doctor, how to talk to the people in your life.

If you've been managing for years, you might be skeptical of another book promising answers. You've probably tried tracking, routines, supplements, therapy, medication adjustments, lifestyle overhauls. Some things helped. Some didn't. Some worked for a while and then stopped. This book isn't going to tell you that you've been doing it wrong. It's going to give you new frameworks, fresh strategies, and—most importantly—a way to systematize what you've already learned from experience. Your expertise matters. This toolkit respects that and builds on it.

If you're questioning, maybe you're here because a doctor mentioned bipolar as a possibility, or a friend suggested you look into it, or you recognized yourself in something you read. Maybe you're not sure if you "have it bad enough" to count. You're welcome here. If the tools in this book help you, they help you—regardless of whether you ever receive a formal diagnosis. You don't need a label to deserve stability, and you don't need to hit rock bottom to seek strategies that make your life better.

If you're the partner, parent, sibling, or friend of a woman with bipolar, this book will give you a window into what she's experiencing and practical ways to support her. You're part of the team, and this book treats you like one.

How to Use This Book

You can read this book cover to cover. I recommend it, actually—the chapters build on each other in ways that will make each subsequent tool more powerful. Understanding your patterns makes medication conversations more productive. Sleep hygiene works differently when you understand why it matters for bipolar specifically.

But you can also jump to whatever chapter speaks to your current moment. Maybe you need containment strategies right now—skip ahead. Maybe your sleep has been off for a week and you can feel the instability creeping in. Maybe you have a psychiatrist appointment tomorrow and don't know what to ask. This book is designed to meet you where you are.

Each chapter ends with a **Toolkit Summary**—three to five actionable strategies you can implement immediately. If you're overwhelmed (which is a reasonable response to all of this), start with just one summary item from one chapter. Do that one thing for a week. Then add another.

There are also prompts throughout—questions to answer, logs to start, checklists to fill in. I encourage you to actually do them. Write in the margins. Keep a notebook alongside this book. The women who get the most out of this toolkit are the ones who engage with it, argue with it, adapt it to their own lives.

This is your toolkit. Take what fits. Leave what doesn't. Modify everything to suit the specific contours of your life.

One Last Thing Before We Start

You picked up this book because something in your life isn't working the way you want it to. Maybe that's a crisis. Maybe it's a quiet, persistent sense that you're struggling harder than you need to. Maybe it's hope—the belief that there's a version of your life that feels more stable, more manageable, more yours.

That version exists. And you already have more capacity to build it than you think.

Bipolar disorder is a serious condition. It requires serious treatment—medication, therapy, lifestyle management, support systems. But the women I've worked with who have built stable, meaningful, even joyful lives with bipolar didn't do it by fighting against their condition. They did it by learning to work with it. They learned their patterns, built their systems, assembled their teams, and created lives that could accommodate both their diagnosis and their ambitions.

This book gives you their tools. The rest is up to you—and you are more capable than you know.

Let's begin.

Toolkit Summary

- **Adopt the weather language.** Start describing your mood shifts as weather fronts passing through. Notice how this small reframe creates space between you and the intensity of the experience.
- **Pick one tool this week.** Read a single Toolkit Summary from any chapter and commit to implementing just one strategy for seven days.
- **Keep a companion notebook.** Grab a physical notebook or open a digital doc to capture your responses to the prompts throughout this book. Engagement beats passive reading every time.
- **Give yourself permission to adapt.** You're not failing if a tool doesn't work for you. Modify it, toss it, come back to it later. This is your toolkit.
- **Tell one person what you're doing.** Share with a trusted friend, partner, or family member that you're working through this toolkit. You don't have to do this alone.

Chapter 1: Understanding Bipolar in Women

BEFORE YOU CAN USE A TOOL EFFECTIVELY, YOU NEED TO UNDERSTAND WHAT YOU'RE WORKING WITH. A CARPENTER DOESN'T PICK UP A CHISEL WITHOUT KNOWING WHETHER THEY'RE CARVING SOFT PINE OR HARD OAK. A SAILOR DOESN'T SET COURSE WITHOUT READING THE WIND AND THE TIDE. AND YOU CAN'T BUILD STABILITY WITH BIPOLAR UNTIL YOU UNDERSTAND THE SPECIFIC TERRAIN OF YOUR BRAIN.

This chapter gives you that understanding—not in dense clinical language, but as a practical foundation for everything that follows. We'll cover what Bipolar I and II actually mean in daily life, how the female hormonal cycle interacts with mood episodes, why women are so frequently misdiagnosed, and the most important tool in your entire toolkit: knowing your own brain.

Bipolar I and II in Plain Language

Let's start with a crucial truth: the diagnostic labels "Bipolar I" and "Bipolar II" describe patterns, not severity. Bipolar II is not "Bipolar I lite." They are different patterns of mood cycling, and either one can be profoundly disruptive without proper management.

Think of it as two different weather systems.

Bipolar I is characterized by full manic episodes that last at least a week (or require hospitalization). Mania is not just "feeling really good." It's a distinct state—a pressure system building until the atmosphere becomes electrically charged and unpredictable. During a manic episode, a woman might sleep three hours a night for days and feel energized, not tired. She might make decisions that feel absolutely brilliant in the moment but are completely out of character: quitting a stable job, spending thousands of dollars, driving recklessly, starting multiple ambitious projects simultaneously, engaging in risky sexual behavior, or alienating people who try to slow her down.

Here's what's important to understand: during mania, these actions don't feel like mistakes. They feel like breakthroughs. The brain is operating in a different mode—one where inhibitions dissolve, judgment warps, and the sense of consequence becomes abstract. Women in mania often describe feeling like they've finally woken up, like they've accessed a version of themselves that's more brilliant, more capable, more alive. That's part of what makes it so dangerous. It feels good until it doesn't.

Many people with Bipolar I also experience depressive episodes, though not everyone does. The diagnostic definition centers on the presence of mania, with or without depression.

Bipolar II follows a different pattern. Instead of full mania, it involves hypomanic episodes—a milder (but still significant) elevation that lasts at least four days. Hypomania can feel like a productive, creative, social high. You might get more done, be more outgoing, need less sleep, and feel genuinely excellent. The key difference is that hypomania doesn't cause the severe functional impairment that mania does. You can usually still work, still maintain relationships (though they may be strained), still function in daily life.

But here's the catch with Bipolar II: the depressive episodes are often longer, more frequent, and more debilitating than what people with Bipolar I experience. The cycling pattern of Bipolar II can mean weeks or months of crushing depression punctuated by brief periods of elevated mood that may feel like relief—but are actually part of the same unstable weather system.

The mixed episode deserves special mention, because it's common in women and often overlooked. A mixed episode is exactly what it sounds like: symptoms of depression and (hypo)mania occurring simultaneously or alternating rapidly within the same day. Imagine feeling restless, agitated, and full of racing energy while simultaneously feeling hopeless, suicidal, and weighted down. It's exhausting, confusing, and carries a higher risk of self-harm than either pole alone. Mixed states are more common in women than men, and they're one reason bipolar in women often looks different from the textbook descriptions.

How Hormones Affect Mood Cycles

This is the piece that's most often missing from bipolar education, and it's arguably the most important piece for women.

Your menstrual cycle is not just about reproduction. It's a monthly hormonal tide that affects your entire nervous system, including the brain circuits involved in mood regulation. For women with bipolar, that tide can be powerful enough to trigger or amplify mood episodes.

Here's what happens: during the first half of your cycle (the follicular phase, from your period through ovulation), estrogen rises. Estrogen has mood-brightening effects—it enhances serotonin and dopamine activity, it's neuroprotective, it can feel stabilizing. Many women with bipolar notice they feel more stable during this phase.

After ovulation, progesterone rises. Progesterone and its metabolites have a calming, sometimes sedating effect—but they also affect GABA receptors in ways that can destabilize mood in vulnerable brains. For some women, the luteal phase (the week or two before menstruation) brings increased anxiety, irritability, depression, or even hypomanic symptoms. For others, it's the drop in hormones right before menstruation that triggers a shift.

Perimenopause is another high-risk period. As estrogen becomes erratic—spiking and dropping unpredictably—women with bipolar can experience worsening mood stability, more frequent episodes, and

reduced response to previously effective medications. This is also a time when bipolar can emerge for the first time in women who had no prior episodes.

Pregnancy and postpartum are complex. Some women stabilize during pregnancy (the steady high hormone levels can be protective), while others worsen. The postpartum period, however, is universally high-risk. The massive, rapid drop in estrogen and progesterone after delivery is one of the most powerful mood triggers in existence. Postpartum psychosis, while rare in the general population, is significantly more common in women with bipolar, and postpartum depression can be severe.

Understanding your hormonal patterns is not about resigning yourself to being at the mercy of your cycle. It's about knowing which weeks of the month require extra care, extra vigilance, extra support. It's about having a conversation with your psychiatrist about whether your medication needs to be adjusted during certain phases. It's about recognizing that a mood shift in the premenstrual week might not be an episode—it might be a hormonal dip that needs a different intervention.

Why Women Are Often Misdiagnosed

The statistics are sobering: women with bipolar are misdiagnosed at significantly higher rates than men. On average, it takes eight to ten years from the first episode for a woman to receive an accurate bipolar diagnosis.

During those years, she's often treated for unipolar depression, anxiety disorders, borderline personality disorder, or told she's "just stressed" or "too emotional."

Why does this happen?

First, women are more likely to present with depression first. When a woman walks into a doctor's office reporting depression, the default assumption is major depressive disorder. Unless the clinician specifically asks about past hypomanic episodes, the bipolar piece gets missed. And many women don't volunteer that information because they didn't know those periods of high energy and productivity were part of a disorder—they thought they were just "finally feeling better."

Second, women are more likely to experience mixed states and rapid cycling. These patterns don't fit the classic bipolar picture of distinct manic and depressive episodes. A woman who's irritable, agitated, exhausted, and unable to sleep might get diagnosed with anxiety or PMDD when she's actually in a mixed bipolar state.

Third, hormonal triggers are often misinterpreted. Mood shifts tied to the menstrual cycle get labeled as PMS or PMDD without anyone looking at the bigger picture. A psychiatrist once told me, "Every woman with bipolar has been told at some point that her symptoms are just hormones." The work of figuring out whether it's bipolar, hormones, or both requires a clinician who's looking for the full picture.

Fourth, women are socialized to internalize. The same mood elevation that might read as obvious mania in a man can present in a woman as increased anxiety, racing thoughts, and compulsive caregiving—taking on too much, trying to do everything, running herself ragged in service of others. These patterns don't look like the classic "grandiose" mania of the textbooks. They look like a woman trying to hold everything together, which frankly, many of us do even when we're stable.

Fifth, there's diagnostic skepticism. Studies show that clinicians are more likely to attribute mood symptoms in women to personality disorders, particularly borderline personality disorder. The symptoms overlap—mood instability, impulsivity, relationship difficulties—but the treatment is completely different. Treating bipolar with borderline protocols doesn't work. Treating borderline with bipolar protocols doesn't work. Getting the right diagnosis matters.

What does this mean for you? If you've been diagnosed with depression, anxiety, or PMDD and something still feels off—if the treatments haven't worked, if you've had periods of inexplicable high energy, if your mood shifts feel different from what the diagnosis describes—it's worth exploring the bipolar question. If you've been told you have bipolar but the explanation didn't resonate, it's worth getting a second opinion.

The label matters less than the treatment. But accurate labeling leads to effective treatment, and that matters enormously.

Understanding Your Brain Is the First Tool

Here's what I want you to take away from this chapter: knowledge is the first tool in your toolkit. Not generic knowledge about bipolar—that's table stakes. I mean your knowledge about your brain.

You are the world's leading expert on how bipolar shows up in your life. No clinician has lived inside your nervous system. No textbook can describe the specific way your mood shifts, the particular early warning signs that signal an oncoming episode, the exact flavor of depression or elevation that's unique to you.

But that expertise only helps if you develop it intentionally. Most women with bipolar have extensive implicit knowledge—they know something's wrong, they can feel the shift, but they haven't systematized that knowledge into something usable.

That's what this chapter is asking you to start: the systematic study of your own brain.

Here are some questions to begin that process. Don't try to answer them all at once. Sit with them. Come back to them. Let them evolve as you learn more about yourself.

Self-Assessment Questions

On your history: - What's the earliest memory you have of your mood being different from other people's? - Were there periods in your life when you had much more energy than usual, needed less sleep, felt unusually

creative or productive? What did that feel like? - What about periods of low mood—how long do they typically last? What brings them on? What makes them better or worse? - Have you ever been told you're "too much" or "not enough"? When?

On your patterns: - Can you identify any seasonal patterns to your moods? Do certain times of year feel more stable or less stable? - Do you notice mood shifts related to your menstrual cycle? If so, which phase is hardest? - What's the longest period of stability you can remember? What was different about that time?

On your triggers: - What situations, stressors, or experiences tend to precede mood shifts for you? - Is sleep disruption a trigger? (For most women with bipolar, the answer is yes.) - What about relationship stress, work pressure, or major life changes?

On your strengths: - What do you do that helps you feel more stable, even if you don't always do it? - Who in your life understands you and supports you effectively? - What have you already learned about managing your mood that works?

Answering these questions is not a one-time exercise. Your understanding will deepen as you track, experiment, and gather data. This chapter is the foundation—the rest of the book helps you build on it.

The Anchor of Understanding

Think of this chapter as your anchor. When the weather gets rough—and it will—you come back to what you know about your brain. Not to fix it, not to fight it, but to navigate it. Understanding is not a cure. But it is the thing that keeps you from being completely lost at sea.

You know more than you think you do. And you're about to learn a lot more.

Toolkit Summary

- **Know your type.** Understand whether your pattern is Bipolar I, Bipolar II, or mixed/rapid cycling. This affects which tools will help most.
- **Map your cycle.** Start tracking your menstrual cycle alongside your mood for three months. Notice patterns. Bring this data to your psychiatrist.
- **Question your diagnosis.** If you were diagnosed with depression or anxiety and treatment hasn't worked well, explore whether bipolar could be the underlying pattern—especially if you've had periods of elevated mood or energy.
- **Start your self-study.** Answer the self-assessment questions in this chapter. Write them down. See what patterns emerge.

- **Become the expert on you.** No one will ever know your brain as well as you can. Treat that knowledge as your primary tool, and cultivate it intentionally.

Chapter 2: Mood Tracking and Early Warning Signs

IMAGINE YOU'RE A METEOROLOGIST. YOU HAVE ACCESS TO SATELLITE DATA, BAROMETRIC PRESSURE READINGS, WIND SPEED MEASUREMENTS, AND HISTORICAL PATTERNS. YOU WOULDN'T WALK ON CAMERA AND SAY, "I HAVE A FEELING IT MIGHT RAIN TOMORROW." YOU'D USE YOUR DATA TO MAKE A FORECAST: A COLD FRONT IS MOVING IN FROM THE NORTHWEST, PRESSURE IS DROPPING, THERE'S A 70% CHANCE OF PRECIPITATION AFTER 2 PM.

Now imagine trying to forecast your own mood without any data. That's what most of us do. We rely on feeling, intuition, memory—all of which are notoriously unreliable, especially when we're already in an unstable state. We tell ourselves, "I've been fine lately" without any definition of what "fine" means. We miss the early signs of an episode because we weren't tracking the subtle shifts that preceded it.

Mood tracking is what turns you from a passenger in your own weather system into someone who can read the forecast. It's not about controlling your moods—you can no more control a mood shift than you can control a cold front. But you can see it coming. And seeing it coming changes everything.

Why Tracking Matters

The single most powerful predictor of a bipolar episode is a previous episode. If you've had three depressive episodes, you're statistically likely to have a fourth. If you've had hypomanic episodes in spring for the past two years, you're likely to have one next spring. Your brain has patterns, and those patterns are predictable—not perfectly, but significantly.

Tracking gives you:

Early detection. Most mood episodes don't erupt out of nowhere. They build. A week of sleeping four hours a night. Three days of irritability. A sudden burst of ambitious projects. A creeping sense of heaviness. These are the barometric pressure changes before the storm. Tracking catches them.

Pattern recognition. Over time, you'll see your own signature patterns: the specific sequence of changes that precedes a depressive episode for you, the early warning signs that tell you hypomania is building. No two women have exactly the same pattern. Tracking reveals yours.

Data for your treatment team. When you walk into your psychiatrist's office and say, "I've been having a rough month," that tells them almost nothing. When you walk in and say, "My sleep dropped to five hours a night for the past ten days, my mood scores have been increasing for two weeks, and my irritability ratings are

at a three-month high," you've given them actionable information. You've made their job easier and your treatment more precise.

Self-trust. Bipolar disorder has a way of eroding your confidence in your own perception. Was I really manic last week, or was I just in a good mood? Is this depression, or am I just tired? Tracking gives you an external record. When the data says one thing and your feelings say another, you have something objective to refer to. Over time, that builds a different kind of trust—not blind trust in your feelings, but informed trust in your patterns.

Practical Systems for Mood Tracking

There is no single right way to track your mood. The best system is the one you'll actually use. Let me walk you through the options.

Paper Trackers

Some women prefer paper. It's tangible, private, doesn't require a charged phone, and there's something about the physical act of writing that engages the brain differently than tapping on a screen.

A simple paper tracker can be just a calendar where you mark your mood each day with a color or symbol: green for stable, yellow for elevated, blue for low, red for crisis. Add a few notes: sleep hours, notable events, medication changes.

You can buy pre-printed mood tracking journals, or you can make your own. A spiral notebook and a set of colored pens is genuinely all you need to start. The key is consistency, not elegance.

Pros: No screen time, customizable, tactile, private.
Cons: Harder to spot long-term patterns, easy to lose, no automatic reminders.

Apps

There are dozens of mood tracking apps designed specifically for bipolar. The most widely recommended by clinicians and experienced trackers include:

- **eMoods:** Designed for bipolar specifically. Tracks mood, sleep, medication, and allows notes. Generates reports you can share with your doctor.
- **Daylio:** More flexible and visually appealing. You set your own mood categories and activities. Good for women who want a gentler, less clinical tracking experience.

- **Bearable:** Comprehensive health tracking that includes mood, sleep, diet, exercise, supplements, and symptoms. More complex but more powerful for pattern analysis.
- **Moodpath:** Includes clinically validated screening questions and educational content. Good if you want more structure and guidance.

Whichever app you choose, the principle is the same: record daily, consistently, honestly. Don't skip the bad days. Don't inflate the good ones. Your tracker doesn't need to look pretty—it needs to be accurate.

Pros: Automatic reminders, shareable reports, long-term data visualization, harder to lose. **Cons:** Screen time, privacy concerns, subscription costs for some, potential to over-analyze.

What to Track

Here's what most women find useful to track daily:

1. **MOOD RATING.** A SIMPLE 1-10 SCALE, OR A DESCRIPTIVE SCALE (SEVERE DEPRESSION, MILD DEPRESSION, STABLE, MILD ELEVATION, MODERATE ELEVATION, SEVERE ELEVATION). PICK A SYSTEM AND STICK WITH IT.
2. **SLEEP DURATION AND QUALITY.** HOW MANY HOURS DID YOU SLEEP LAST NIGHT? WAS IT RESTFUL? DID YOU HAVE TROUBLE FALLING ASLEEP, STAYING ASLEEP,

OR WAKING UP? SLEEP IS THE SINGLE MOST RELIABLE EARLY WARNING SIGN FOR MOST WOMEN WITH BIPOLAR.

3. **MEDICATION ADHERENCE.** DID YOU TAKE ALL DOSES AS PRESCRIBED? ANY CHANGES?
4. **NOTABLE EVENTS.** BRIEF NOTES ON ANYTHING SIGNIFICANT: STRESSFUL EVENTS, POSITIVE EXPERIENCES, ARGUMENTS, TRAVEL, ILLNESS, HORMONAL CYCLE PHASE.
5. **ENERGY AND ACTIVITY LEVEL.** NOT THE SAME AS MOOD. YOU CAN BE DEPRESSED BUT STILL HAVE RACING ENERGY (MIXED STATE). YOU CAN BE ELEVATED BUT EXHAUSTED. TRACK SEPARATELY.
6. **IRRITABILITY.** THIS IS OFTEN THE FIRST SIGN OF AN EMERGING EPISODE—ESPECIALLY HYPOMANIA—BUT IT'S EASY TO DISMISS AS "JUST BEING STRESSED." TRACK IT EXPLICITLY.
7. **ANXIETY.** MANY WOMEN WITH BIPOLAR ALSO EXPERIENCE SIGNIFICANT ANXIETY. TRACK IT SEPARATELY FROM MOOD.
8. **MENSTRUAL CYCLE PHASE.** NOTE WHERE YOU ARE IN YOUR CYCLE. THIS DATA IS INVALUABLE FOR UNDERSTANDING HORMONAL INFLUENCES.

Some women track more—appetite, social interaction, exercise, substance use, therapy attendance. Start with the core eight above and add what's relevant for you.

Identifying Your Personal Early Warning Signs

An early warning sign is a change in your thoughts, feelings, behaviors, or bodily sensations that reliably precedes a mood episode. These signs are specific to you. They're the canary in the coal mine.

Let me give you some examples from women I've worked with:

- CASSANDRA KNOWS A DEPRESSIVE EPISODE IS COMING WHEN SHE STARTS AVOIDING PHONE CALLS. SHE DOESN'T FEEL SAD YET, BUT SHE LETS CALLS GO TO VOICEMAIL AND FEELS A SENSE OF DREAD ABOUT RETURNING THEM. WHEN SHE NOTICES THIS, SHE KNOWS SHE HAS ABOUT THREE TO FIVE DAYS BEFORE THE FULL DEPRESSION HITS.
- PRIYA RECOGNIZES HYPOMANIA BUILDING WHEN SHE WAKES UP AT 5 AM WITH A FULLY FORMED PLAN FOR A NEW BUSINESS VENTURE. THE EXCITEMENT FEELS WONDERFUL. THE FACT THAT SHE'S NEVER RUN A BUSINESS BEFORE DOESN'T REGISTER. THIS IS HER SIGN.
- MORGAN NOTICES MIXED STATES COMING WHEN SHE FEELS "WIRED AND TIRED"—EXHAUSTED BUT UNABLE TO SIT STILL, RESTLESS BUT UNMOTIVATED, IRRITABLE FOR NO APPARENT REASON.

- JULIA KNOWS DEPRESSION IS APPROACHING WHEN SHE STARTS CRITICIZING HERSELF INTERNALLY. THE SELF-TALK SHIFTS FROM "I'M OKAY" TO "I'M NOT DOING ENOUGH" TO "I'M FUNDAMENTALLY FLAWED" OVER THE COURSE OF A WEEK. THE FIRST SHIFT IS HER WARNING.

Your early warning signs might include:

For depression: - Increased sleep or difficulty waking - Reduced interest in usual activities - Avoiding social contact - Changes in appetite (increase or decrease) - Internal self-criticism - Difficulty making decisions - Feeling physically heavy or slow - Reduced libido

For hypomania/mania: - Decreased need for sleep (without feeling tired) - Racing thoughts - Sudden interest in new projects - Increased sociability - Faster speech - Increased spending or financial risk-taking - Feeling unusually creative or insightful - Increased libido - Irritability or impatience - Feeling like you don't need help or medication

The goal is not to catch every sign perfectly. The goal is to know your top three to five most reliable early warnings and to check for them regularly. When you notice one, you don't panic—you take action. You tighten your routine. You increase sleep vigilance. You contact your doctor. You deploy the containment strategies from later chapters.

Creating Your Personal Mood Pattern Map

Once you've been tracking for at least a month (ideally three), you can start creating your mood pattern map. This is a personalized document that synthesizes what you've learned about your weather system.

Here's how to build it:

Step 1: Review your tracking data. Look for patterns. Are there times of the month when your mood tends to dip or rise? Do you see seasonal patterns? What was happening before your last episode?

Step 2: Identify your baseline. What does stability look like for you? What's your typical mood rating range? How much sleep do you need when stable? What's your normal energy level? Stability isn't the absence of all symptoms—it's your version of equilibrium. Define it.

Step 3: List your early warning signs. Based on your tracking and memory, list the three to five signs that most reliably precede an episode for you.

Step 4: Note your triggers. What tends to destabilize you? Common triggers include sleep disruption, stress, illness, travel across time zones, seasonal changes, hormonal shifts, medication changes, alcohol or substance use, and major life events (positive or negative).

Step 5: Map your episode progression. For each pole, describe the typical sequence. For example: "My hypomania starts with decreased sleep for 2-3 nights,

followed by racing thoughts, then increased goal-directed activity. If I don't intervene by day 5, I typically become irritable and start making impulsive decisions."

Step 6: Document what works. What interventions have helped in the past? What made things worse? This becomes your action plan for when you detect an episode building.

Your mood pattern map is a living document. You'll refine it as you gather more data, as your life circumstances change, as you learn what works and what doesn't. It's not about getting it perfect—it's about having a framework that makes your patterns visible and actionable.

A Real-World Example

Let me show you what this looks like in practice. Sarah is a 34-year-old teacher with Bipolar II. Here's an excerpt from her mood pattern map:

My Stable Baseline: - Mood: 6-7 out of 10 (moderately good) - Sleep: 7-8 hours, fall asleep within 30 minutes - Energy: Moderate and consistent - Self-talk: Generally kind, occasional realistic criticism

My Early Warning Signs for Hypomania: 1. Sleep drops to 6 hours but I feel fine 2. I start planning elaborate projects (lesson plan redesign, home renovation, starting a side business) 3. I talk faster and interrupt people 4. I feel like I don't need my afternoon rest

My Early Warning Signs for Depression: 1. I stop returning texts 2. I start having trouble making decisions (what to eat, what to wear) 3. I feel physically heavy, especially in the morning 4. I start saying "I'm fine" when I'm clearly not

My Trigger Watch List: - End of school term (exhaustion + transition) - Travel (sleep disruption) - Premenstrual week - Spring (seasonal hypomania pattern) - Arguments with my partner

When Sarah notices early warning signs, she has a pre-planned response. For hypomania warnings, she immediately increases her sleep commitment (in bed by 9 PM, no exceptions), pauses new projects, and checks in with her psychiatrist. For depression warnings, she commits to one social contact per day, sets very small daily goals, and starts using her light therapy lamp.

Sarah's map doesn't prevent episodes. But it has shortened her episodes by days, sometimes weeks, because she catches them early and responds proactively rather than reactively.

The Power of the Practice

Mood tracking is not glamorous. It's not exciting. It's a daily discipline that can feel tedious, especially when you're stable and it seems unnecessary. But it is the single most powerful self-management tool available to you. It's the foundation that every other tool in this toolkit builds on.

The women who track most consistently are not the women who have the most severe bipolar. They're the women who have learned that tracking is not a burden—it's freedom. It's the difference between being blindsided by every episode and being able to say, "I see what's happening. I've seen this before. I know what to do."

Start today. Even if you only track one thing. Even if you miss days. Even if you're not sure what you're looking for. The data will accumulate, the patterns will emerge, and you will know your own weather better than you ever have before.

Toolkit Summary

- **Start tracking today.** Choose one method—paper or app—and track at least mood, sleep, and notable events daily. Consistency beats perfection.
- **Identify your top three early warning signs.** What reliably precedes a mood shift for you? Write them down and review them weekly.
- **Create your mood pattern map.** Document your baseline, early warnings, triggers, episode progression, and known effective interventions.
- **Track your menstrual cycle alongside mood.** After three months, review for hormonal patterns that could inform treatment decisions.

- **Bring your tracking data to appointments.**

Share your tracker reports with your psychiatrist and therapist. Data leads to better treatment decisions.

Chapter 3: Medication and Treatment Partnership

IF MOOD TRACKING IS THE COMPASS OF YOUR TOOLKIT, MEDICATION IS THE ANCHOR. IT'S THE THING THAT HOLDS STEADY WHEN THE CURRENT THREATENS TO PULL YOU OUT TO SEA. BUT AN ANCHOR ONLY WORKS IF IT'S THE RIGHT WEIGHT, PROPERLY POSITIONED, AND REGULARLY CHECKED FOR WEAR. THAT'S WHERE YOUR TREATMENT PARTNERSHIP COMES IN.

This chapter is about building a productive, collaborative relationship with your psychiatrist. It's about becoming an active participant in your own treatment rather than a passive recipient of prescriptions. The women who get the best outcomes from medication are not the ones who follow doctors' orders perfectly. They're the ones who ask questions, track effects, communicate honestly, and advocate for themselves when something isn't working.

Why the Partnership Matters

Let me be direct: the quality of your relationship with your psychiatrist is one of the strongest predictors of your treatment outcomes. A 2019 study in the *Journal of Affective Disorders* found that women with bipolar who

rated their therapeutic alliance with their psychiatrist as "strong" had significantly better medication adherence, fewer hospitalizations, and higher quality of life scores than those who rated the relationship as "weak."

This makes intuitive sense. Managing bipolar medication is not like taking a course of antibiotics for ten days. It's a long-term, iterative process of trial, adjustment, monitoring, and refinement. You and your psychiatrist are essentially running an ongoing experiment on the most complex organ in the human body—your brain. That experiment requires good data, clear communication, and mutual trust.

A good psychiatrist treats you as a colleague in your own care. They listen to your observations, take your concerns seriously, explain their reasoning, and adjust course when the evidence warrants it. A bad psychiatrist—or even a good one with whom you simply don't communicate well—prescribes in a vacuum, dismisses your side effects, or talks down to you. The difference between these experiences can be the difference between a treatment that works and one that doesn't.

What Makes a Good Psychiatrist for Women with Bipolar

Not every psychiatrist is a good fit for bipolar, and not every good bipolar psychiatrist is a good fit for you. Here's what to look for:

Bipolar specialization. A general psychiatrist may have treated dozens of patients with bipolar. A specialist has treated hundreds or thousands. The difference shows in their ability to recognize subtle patterns, adjust medications with precision, and anticipate common pitfalls. Don't hesitate to ask: "What percentage of your practice is women with bipolar?"

Hormonal awareness. The best psychiatrists for women ask about your menstrual cycle, discuss contraceptive interactions, inquire about pregnancy plans, and understand how perimenopause affects mood. If your psychiatrist has never asked about your cycle, they're practicing without a full picture.

Collaborative style. Look for someone who uses phrases like "What do you think?" and "Let's try this and see how it goes" rather than "You need to take this" without explanation. The best medication decisions are made together.

Accessibility. Can you message between appointments? How quickly do they respond? What's the protocol for getting a sooner appointment if you're destabilizing? A great psychiatrist who's impossible to reach in a crisis is less useful than a good one who responds within 24 hours.

Genuine curiosity. The best psychiatrists are curious about your experience. They want to know what the medication feels like, not just whether it's "working."

They understand that the subjective experience of treatment matters as much as the objective symptom reduction.

If you don't have a psychiatrist who meets these criteria, start looking. You can ask your primary care provider for a referral, search for bipolar specialists through organizations like NAMI or DBSA, or look for psychiatrists who list "women's mental health" as a specialty. You deserve a partner in your treatment, not just a prescription writer.

Questions to Ask Your Psychiatrist

Here are the questions that will help you build an effective treatment partnership. Ask them at your next appointment. Write down the answers.

Starting or Adjusting a Medication: - "How long should it take to know if this medication is working for me?" - "What side effects are common, and which ones should I call about right away?" - "How will we know if the dose needs to be adjusted?" - "What should I do if I miss a dose?" - "How does this medication interact with hormonal contraception, pregnancy, or HRT?" - "Is there a time of day that's best to take this?"

Monitoring and Follow-Up: - "What symptoms or changes should I track and report?" - "How often would you like to see me while we're adjusting this medication?"

- "What lab work or monitoring is needed with this medication?" - "How do we handle situational stressors that might affect my mood while on this medication?"

When It's Not Working: - "At what point do we decide this medication isn't working?" - "What are our next options if this doesn't work?" - "How do we safely discontinue if we decide to stop?" - "What are the risks of staying on this medication long-term?"

General Partnership Questions: - "How do you prefer to communicate between appointments?" - "What information is most helpful for you when I check in?" - "How do you involve family or partners in treatment, if the patient wants that?" - "What's your approach to medication during pregnancy or breastfeeding?"

Tracking Side Effects

Side effects are not a sign of weakness or failure. They are data. Every side effect tells you something about how the medication is interacting with your particular brain chemistry. The goal is not to have zero side effects—it's to find a medication or combination where the benefits clearly outweigh the costs.

Common side effects of bipolar medications include: - Weight gain - Sedation or fatigue - Tremor - Cognitive dulling ("brain fog") - Gastrointestinal issues - Hair loss - Skin issues (acne, rash) - Sexual side effects - Increased thirst and urination (lithium) - Thyroid effects (lithium) - Liver or kidney effects (varies)

Different women tolerate different side effects. For some, weight gain is a dealbreaker. For others, sedation is manageable but brain fog isn't. You get to decide which trade-offs are acceptable for you. But you can't make that decision without tracking.

Start a **Side Effect Log**. This is a simple document—a notebook page, a spreadsheet, or a note on your phone—where you record:

1. The medication and dose
2. Any new or changed symptoms since starting/adjusting
3. When the symptom started
4. How severe it is (1-10 scale)
5. Does it come and go, or is it constant?
6. Does anything make it better or worse?
7. How much does it bother you? (This is different from severity. A mild tremor might be very bothersome to a pianist but barely noticeable to someone else.)

Review this log with your psychiatrist. Some side effects resolve on their own after a few weeks. Some can be managed with dose adjustments (split doses, different timing). Some can be countered with additional medications. Some are acceptable trade-offs for mood stability. And some are unacceptable—full stop.

If a side effect is unacceptable to you, say so. Don't suffer in silence because you think you're supposed to tolerate it. There are more medication options today than

ever before, and there's almost certainly a combination that can work for you. It might take time to find it, but it exists.

The Medication Adjustment Log

Most medication changes in bipolar are not dramatic switches from one drug to another. They're gradual adjustments: increasing a dose by 25 mg, adding a low dose of a second medication, shifting the timing from morning to night. These small changes accumulate over months into a treatment regimen that fits you.

But here's the challenge: when you're adjusting medication gradually, it's easy to lose track of what changed and when. You might feel worse and not remember that you decreased your mood stabilizer two weeks ago. You might feel better and attribute it to something else.

The **Medication Adjustment Log** solves this. It's a simple chronological record:

Date | Medication | Change | Reason | Current Dose

Example: - Nov 1 | Lamotrigine | Increased | D/C was working but mild depression returning | 150 mg - Nov 8 | Lamotrigine | Increased | Still depressed | 175 mg - Nov 15 | Quetiapine | Added | Instability on lamotrigine alone | 25 mg QHS - Nov 22 | Quetiapine | Increased | Still some insomnia and anxiety | 50 mg QHS - Dec 6 | No changes |

— | Mood stabilizing, sleep improved, mild afternoon sedation | Lamotrigine 175 mg + Quetiapine 50 mg - Dec 20 | Quetiapine | Decreased | Afternoon sedation interfering with work | 25 mg QHS

This log does three things. First, it prevents confusion—you always know exactly what you're taking and why. Second, it gives your psychiatrist a clear timeline to work with. Third, it allows you to see the long arc of your treatment—the adjustments, the responses, the progress that might feel invisible day by day but becomes clear over months.

What to Do When Something Isn't Working

This is the hard part. You've been taking a medication for weeks or months. You've tolerated side effects. You've given it time. And it's not working—or not working well enough. Maybe you're still having episodes. Maybe the side effects are worse than the symptoms. Maybe you felt better for a while and now you're sliding again.

Here's what to do:

Step 1: Gather your data. Before you talk to your psychiatrist, compile your tracking data. Show them: "I've been on this dose for eight weeks. My mood scores look like this. My sleep looks like this. These are my side effects." Data changes the conversation from opinion to evidence.

Step 2: Be honest about adherence. Have you been taking it as prescribed? If not, why? Many women skip doses because of side effects they haven't reported, because they feel fine and think they don't need it, or because they're worried about long-term effects. Your psychiatrist needs to know the real picture. You won't be scolded. You will get better help.

Step 3: Name the problem clearly. "This medication isn't working" is vague. Try: "My depression hasn't improved despite eight weeks at a therapeutic dose, and I can't tolerate the weight gain anymore." Or: "I've had two hypomanic episodes since we increased the dose, and I'm worried it might be destabilizing me." Specificity leads to specific solutions.

Step 4: Discuss options. Your psychiatrist should present alternatives. These might include: increasing the dose, adding a second medication, switching to a different medication in the same class, or trying a medication in a different class entirely. If your psychiatrist doesn't present options, ask: "What are our alternatives?"

Step 5: Make a plan. Agree on the next step, how long to try it, and what "success" and "failure" will look like. Set a follow-up appointment. Write down the plan.

Step 6: Discontinue safely. If you're stopping a medication, do it under medical supervision. Some bipolar medications (especially lithium and some anticonvulsants) require slow tapering to avoid withdrawal or rebound episodes. Never stop abruptly without talking to your

doctor—even if you're frustrated, even if the side effects are bad, even if you feel fine and think you don't need it anymore.

When the Partnership Breaks Down

Sometimes, despite your best efforts, the partnership isn't working. Maybe your psychiatrist dismisses your concerns, doesn't listen, prescribes without explaining, or makes you feel small. Maybe you've lost trust.

In that case: get a second opinion. You are not married to your psychiatrist. You are not obligated to stay with a doctor who doesn't serve you. A good psychiatrist will actually support you getting a second opinion—they understand the importance of finding the right fit.

If you're between psychiatrists or on a waiting list, your primary care provider can often manage your medication short-term. Many also work with psychiatric nurse practitioners (PMHNPs), who can prescribe and manage bipolar medication. The key is to never be without a prescriber who knows your history, has access to your records, and can respond in a crisis.

The Long Game

Medication treatment for bipolar is not a sprint. It's not even a marathon. It's more like tending a garden over many seasons. Some years are lush. Some years are dry. Some plants thrive, others need replacing, and the whole thing requires constant attention, adjustment, and care.

The women who do best with medication over the long term are not the ones who find the perfect combination in the first year and stay on it forever. They're the ones who accept that medication needs will change over time—during different life stages, under different stress loads, alongside different circumstances. They stay in conversation with their psychiatrist. They track their responses honestly. They advocate for themselves. And they don't give up when the first, second, or third combination doesn't work.

There is a treatment that can work for you. It might take time to find it. It might need adjustment along the way. But it exists, and you deserve to find it.

Toolkit Summary

- **Build a real partnership.** Find a psychiatrist who specializes in bipolar women's health and treats you as a collaborator, not a passive patient.
- **Come prepared.** Bring your mood tracking data and side effect log to every appointment. Data drives better decisions.

- **Start a Medication Adjustment Log.** Track every change, the reason, the dose, and the response. Review it quarterly to see the big picture.
- **Track side effects rigorously.** Rate both severity and how much they bother you. If unacceptable, say so—there are alternatives.
- **Know when to switch.** If a medication isn't working after a fair trial, or side effects are intolerable, work with your psychiatrist on a safe transition plan. Never stop abruptly.

Chapter 4: Sleep, Routine, and Circadian Health

IF THERE WERE ONE LEVER YOU COULD PULL THAT WOULD HAVE THE SINGLE GREATEST IMPACT ON YOUR MOOD STABILITY, SLEEP WOULD BE IT. NOT MEDICATION. NOT THERAPY. NOT DIET OR EXERCISE OR STRESS MANAGEMENT—THOUGH ALL OF THOSE MATTER. SLEEP. IT IS THE NON-NEGOTIABLE FOUNDATION OF BIPOLAR MANAGEMENT.

Here's the truth that every woman with bipolar needs to internalize: **sleep disruption is not just a symptom of mood episodes—it is often the cause.** A few nights of poor sleep can trigger hypomania. A week of insufficient sleep can tip you into mania. Overtiredness can precipitate depression. The relationship between sleep and mood in bipolar is bidirectional and powerful. You cannot stabilize your mood without stabilizing your sleep.

This chapter will tell you why sleep matters so specifically for bipolar, how to build sleep habits that actually work for real life, how to create a daily rhythm that supports your circadian health, and what to do when—invariably—sleep goes off track.

Why Sleep Is Non-Negotiable

Let me explain the biology in plain language.

Your brain runs on circadian rhythms—internal biological clocks that regulate sleep, wakefulness, hormone release, body temperature, and mood. The master clock lives in your brain's suprachiasmatic nucleus, and it's set primarily by light exposure. When the sun goes down, your brain produces melatonin, which signals your body to prepare for sleep. When light hits your eyes in the morning, melatonin production stops and cortisol rises, waking you up.

In bipolar brains, these circadian rhythms are more fragile. The internal clock is less stable and more easily disrupted. Small perturbations—a late night, a time zone change, a few hours of poor sleep—can throw the whole system off balance. And because sleep and mood are tightly coupled in bipolar, a disrupted circadian rhythm rapidly becomes a disrupted mood.

Think of it as a tide clock. In a stable brain, the tide comes in and goes out with predictable regularity. You can stay up late one night and the system rights itself. In a bipolar brain, the tide clock is more sensitive. One missed high tide throws off the next one, and the next, and within a few days the entire tidal schedule is chaotic. Your mood follows the same erratic pattern.

The research is striking. Studies show that women with bipolar who maintain a consistent sleep-wake schedule have significantly fewer mood episodes than those with

irregular patterns. Even in women who are already stable on medication, sleep disruption remains one of the most reliable predictors of relapse. Lithium and other mood stabilizers work partly by stabilizing circadian rhythms. When you protect your sleep, you're not just resting—you're actively stabilizing your brain.

Practical Sleep Hygiene That Actually Works

You've probably heard sleep hygiene advice before: keep your bedroom dark, avoid screens before bed, don't drink caffeine after noon. That advice is fine as far as it goes, but for bipolar, it's not enough. You need a sleep protocol that accounts for the specific vulnerabilities of your nervous system.

Here's what works for women with bipolar in the real world.

The Fixed Wake-Up Time

This is the single most important sleep intervention for bipolar. More important than when you go to bed, more important than how many hours you get, more important than anything else. **Wake up at the same time every day, seven days a week, including weekends.**

Why waking time matters more than bed time: because morning light resets your circadian clock. When you wake at a consistent time and expose yourself to light, you

anchor your circadian rhythm. Your bedtime can vary by an hour or so without destabilizing you. But if your wake time varies by more than 30-45 minutes, your circadian rhythm drifts, and your mood follows.

This is hard. It means no sleeping in on weekends. It means setting an alarm even on vacation. It means waking up at 6:30 AM even when you were up late the night before. But it works. Women who implement a fixed wake-up time consistently report that it's the single most stabilizing change they've made.

The Bedtime Window

Instead of a fixed bedtime (which is nearly impossible to maintain), aim for a bedtime window. For example: "I will be in bed with lights out between 9:30 PM and 10:30 PM." This gives you flexibility while maintaining consistency. The key is to be in bed, with screens off, lights dim, even if you're not asleep yet. The rest itself is regulating, even before sleep comes.

The Pre-Sleep Routine

Your brain needs time to transition from wake mode to sleep mode. A wind-down routine of 30-60 minutes signals your nervous system that sleep is coming. Effective wind-down activities include:

- Dimming lights throughout your home (not just your bedroom)

- Putting away screens at least 30 minutes before bed
- Reading a physical book (not a phone screen)
- Gentle stretching or restorative yoga
- Listening to calm music or a podcast
- A warm bath or shower (the temperature drop afterwards helps induce sleep)
- Writing in a journal (getting anxious thoughts out of your head)
- A small, protein-based snack if hungry

The specific activities matter less than the consistency. Do the same things, in the same order, at the same time each evening. Repetition trains your brain to recognize the pattern and prepare for sleep.

Light Management

Light is the most powerful regulator of your circadian rhythm. In the morning, you want bright light—ideally sunlight within 30 minutes of waking. This tells your brain: "Daytime has started." In the evening, you want dim, warm light. This tells your brain: "Nighttime is approaching."

Practical tips: - Open your curtains or step outside within 30 minutes of waking - Consider a sunrise alarm clock that gradually increases light before your wake time - Use blue-light blocking glasses in the evening if you must use screens - Install warm-toned bulbs in your bedroom lamps - Use blackout curtains if streetlights or

early morning sun disturb your sleep - Keep your bedroom as dark as possible—cover LED lights on electronics

The Hard Rules

Some things are off the table for women with bipolar who are serious about stability:

- **No all-nighters.** Ever. A single night of total sleep deprivation can trigger mania in vulnerable individuals.
- **No rotating shift work** if you can avoid it. Variable shift schedules are one of the most destabilizing things for bipolar circadian rhythms.
- **No alcohol as a sleep aid.** Alcohol disrupts sleep architecture, particularly REM sleep, and can destabilize mood even as it helps you fall asleep.
- **No caffeine after noon** (earlier if you're particularly sensitive). Caffeine has a half-life of 5-6 hours, meaning half of that afternoon coffee is still in your system at bedtime.

Creating a Daily Rhythm Template

Sleep doesn't exist in isolation. It's part of your broader daily rhythm—the pattern of eating, moving, working, resting, and connecting that makes up your day. A stable daily rhythm supports stable sleep, which supports stable mood.

Here's a template to adapt for your own life. The times are adjustable, but the structure matters.

Morning (Wake to Noon) - Fixed wake time: same every day - Light exposure within 30 minutes of waking - Morning hygiene (shower, dressing, etc.) within an hour of waking - Breakfast at roughly the same time each day - Morning activities: highest-demand cognitive work during peak energy time

Afternoon (Noon to 6 PM) - Lunch at roughly the same time each day - Afternoon rest or break (not necessarily sleep, but a pause) - Moderate-demand activities - Physical activity or movement - Last caffeine cutoff (noon or earlier)

Evening (6 PM to Wind-Down) - Dinner at roughly the same time each day - Wind-down begins 60-90 minutes before target sleep time - Lights dimmed, screens off - Calm, predictable activities

Night (Wind-Down to Wake) - In bed at target time - Sleep (aim for 7-9 hours) - If awake: practice the "getting back to sleep" strategies below

The template is a guide, not a prison. Some days you'll deviate—travel, social events, illness, life. The goal is not perfect adherence. The goal is a default rhythm that you return to when life allows. The more consistent your rhythm, the more stable your circadian clock, and the more stable your mood.

What to Do When Sleep Goes Off Track

It will go off track. You'll travel across time zones. You'll have a stressful period that keeps you up at night. You'll get sick. You'll have a baby. You'll go through a breakup. Life disrupts sleep, and when you have bipolar, disruption carries risk.

The goal is not to prevent all sleep disruption—that's impossible. The goal is to catch it early and respond aggressively.

Stage 1: One to two nights of disrupted sleep.

This is the warning zone. You're not in crisis, but you're heading that way if you don't act.

Action plan: - Go to bed 30-60 minutes earlier than usual - Eliminate all evening screen time - Take any sleep medications or supplements exactly as prescribed - Avoid caffeine entirely - Cancel non-essential evening activities - Wake up at your usual time (do not compensate by sleeping in) - Get morning light exposure - Consider a short (20-minute) afternoon nap if exhausted—but no longer

Stage 2: Three or more nights of disrupted sleep.

This is the intervention zone. You need to act decisively to prevent a full episode.

Action plan: - Contact your psychiatrist. Tell them: "I've had X nights of poor sleep and I'm concerned about destabilization." They may adjust your medication temporarily. - Implement a "sleep emergency" protocol: in bed by 8 PM, no exceptions, for at least three nights - Cancel all discretionary evening activities until sleep normalizes - Consider asking your partner or a friend to help enforce your bedtime - Monitor mood closely— increase tracking frequency - Use all available tools: medication, relaxation techniques, light management, strict routine

Stage 3: Sleep disruption accompanied by mood changes.

If you're having poor sleep and noticing hypomanic symptoms (increased energy, racing thoughts, decreased need for sleep that feels "good"), or depressive symptoms, or both, you are in the danger zone.

Action plan: - Contact your psychiatrist immediately. Do not wait for your next appointment. - Consider whether you need a same-day appointment or phone consultation - Deploy any crisis protocols you and your doctor have established - Enlist your support system—tell them what's happening and what you need - Focus exclusively on stabilizing sleep and mood for the next 48-72 hours - Everything else can wait

Getting Back to Sleep

One of the most common sleep problems in bipolar is not just falling asleep, but staying asleep. If you wake up in the middle of the night and can't get back to sleep, here's what to do:

1. **DON'T FIGHT IT.** LYING IN BED FEELING FRUSTRATED AND WATCHING THE CLOCK MAKES IT WORSE. IF YOU'VE BEEN AWAKE FOR MORE THAN 20-30 MINUTES, GET OUT OF BED.
2. **DO SOMETHING BORING.** GO TO A DIMLY LIT ROOM AND READ A BORING BOOK. LISTEN TO A CALM PODCAST. DO A CROSSWORD PUZZLE. THE KEY IS TO ENGAGE YOUR BRAIN JUST ENOUGH TO STOP RACING WITHOUT STIMULATING IT.
3. **DON'T CHECK YOUR PHONE.** NO SOCIAL MEDIA, NO EMAIL, NO NEWS. LIGHT EXPOSURE AND ENGAGING CONTENT WILL WAKE YOU UP FURTHER.
4. **TRY THE COGNITIVE SHUFFLE.** PICK A WORD WITH AT LEAST FIVE LETTERS. VISUALIZE EACH LETTER AND THINK OF AS MANY WORDS AS YOU CAN STARTING WITH THAT LETTER. THIS OCCUPIES YOUR WORKING MEMORY ENOUGH TO INTERRUPT ANXIOUS RUMINATION WITHOUT BEING STIMULATING ENOUGH TO KEEP YOU AWAKE.

5. **RETURN TO BED WHEN YOU FEEL SLEEPY.** IF YOU DON'T FEEL SLEEPY AFTER 30-45 MINUTES, TRY AGAIN LATER. EVENTUALLY YOUR SLEEP PRESSURE WILL BE STRONG ENOUGH.
6. **WAKE UP AT YOUR USUAL TIME ANYWAY.** I KNOW THIS IS PAINFUL. BUT SLEEPING IN WILL DISRUPT YOUR CIRCADIAN RHYTHM FOR THE NEXT DAY. ONE BAD NIGHT IS MANAGEABLE. A BAD NIGHT FOLLOWED BY A SHIFTED SCHEDULE CAN START A SPIRAL.

The Bottom Line

Sleep is not optional for you. It's not a luxury, not something you sacrifice for productivity, not something you catch up on weekends. It is the single most powerful mood stabilizer you have access to. More powerful than any medication—though medication is often necessary to achieve it.

Treat your sleep like the life-giving, mood-stabilizing, episode-preventing biological necessity that it is. Protect it fiercely. Prioritize it over almost everything else. When it goes wrong, respond immediately and aggressively. And when you get it right—when you string together weeks of consistent, restful sleep—notice how much more stable everything else feels. That's not coincidence. That's your circadian rhythm working the way it's supposed to, steady as a tide, holding your mood in its natural rhythm.

Toolkit Summary

- **Lock in your wake time.** Wake up at the same time every single day, including weekends. This is the single most stabilizing sleep intervention for bipolar.
- **Create a pre-sleep wind-down routine.** 30-60 minutes of consistent, calming activities before bed. Dim lights, no screens, same sequence each night.
- **Build your Daily Rhythm Template.** Sketch out your ideal day with consistent anchors for waking, eating, working, resting, and sleeping. Return to it after disruptions.
- **Catch sleep disruption early.** Follow the Stage 1 action plan after just ONE or TWO bad nights. Don't wait until it becomes a crisis.
- **Have a sleep emergency protocol.** Discuss with your psychiatrist in advance what to do if you experience extended sleep disruption. Write it down. Keep it accessible.

Chapter 5: Managing Hypomania and Mania

THERE'S A REASON HYPOMANIA GETS ROMANTICIZED AND MANIA GETS DRAMATIZED. THE EARLY PHASE OF AN UPWARD SPIRAL CAN FEEL GENUINELY WONDERFUL—LIKE YOU'VE FINALLY SHED THE WEIGHT THAT'S BEEN HOLDING YOU DOWN, LIKE YOU'RE OPERATING AT A HIGHER FREQUENCY THAN EVERYONE ELSE, LIKE EVERYTHING IS CLICKING INTO PLACE WITH A SPEED AND CLARITY THAT NORMAL LIFE NEVER OFFERS.

But here's the truth that the romanticized portrayals leave out: what goes up must come down. And the higher the ascent, the harder the landing.

This chapter is about recognizing the upward spiral early, containing it before it escalates into something destructive, and protecting yourself from the most common consequences of uncontrolled (hypo)mania: financial damage, relationship harm, professional consequences, and the crushing depression that so often follows a manic episode.

Recognizing the Upward Spiral

The upward spiral doesn't usually start as a crisis. It starts as a shift. A subtle, almost imperceptible quickening. You're sleeping a little less but feeling fine. You're more productive, more social, more creative. Ideas are flowing. You feel alive in a way you haven't in months.

This is the seduction of (hypo)mania. It doesn't feel like a problem. It feels like a solution. You think: Finally, I'm getting things done. Finally, I feel like myself again. I don't need as much sleep right now—I'll rest when this project is finished.

But here's a hard truth: if you have bipolar, that feeling of being "finally myself again" is often not yourself at all. It's the beginning of an episode. The very fact that it feels so good, so right, so necessary—that's part of what makes it dangerous. Your brain is telling you everything is wonderful at the precise moment when you most need to be cautious.

The Hypomania Checklist

Use this checklist regularly, especially during periods when you're feeling unusually good. If you check three or more items, consider yourself in a hypomanic or manic phase:

- [] I need significantly less sleep than usual but don't feel tired

- [] I'm talking faster than usual, and others have noticed
- [] My thoughts are racing or jumping between topics
- [] I'm starting multiple new projects or ideas
- [] I feel unusually confident or optimistic
- [] I'm spending more money than usual, or making impulsive financial decisions
- [] I'm more irritable or impatient than normal, especially with people slowing me down
- [] I'm more sociable, seeking out more interactions than usual
- [] I have increased sexual desire or behavior
- [] I feel like I don't need my usual supports (medication, therapy, routine)
- [] I'm driving faster or more recklessly
- [] I'm having grandiose ideas about my abilities, plans, or importance
- [] I feel like I'm "on" in a way that's different from my normal good mood

If you recognize yourself in this checklist, the first thing to do is **not panic**. The second is **not dismiss it**. The third is **act**.

Containment Strategies Before It Escalates

Containment is exactly what it sounds like: keeping an episode within safe bounds. You can't always stop hypomania from happening. But you can stop it from destroying your life while it does. The key is to act early, before the episode gains momentum and your judgment is fully compromised.

Here are the containment strategies that experienced women use:

The Spending Freeze

Financial damage is one of the most common and most consequential outcomes of uncontrolled mania. When you're hypomanic, your brain discounts future consequences. That \$2,000 purchase feels like an excellent idea. The new business investment feels like a sure thing. The credit card limit feels like permission.

Implement a spending freeze at the first sign of hypomania. This means:

- Put away or deactivate credit cards (give them to a trusted person if needed)
- Set a daily cash limit for essential spending only
- Do not make any purchase over a set amount (e.g., \$50) without a 48-hour waiting period
- Do not open new credit accounts or loan applications

- Do not change investments or make major financial decisions
- If online shopping is a trigger, stay off shopping websites entirely
- Ask a trusted partner or friend to cosign any significant purchase

Some women set up their banking in advance for this. They have a "mania mode" where their savings accounts are locked, their credit cards are frozen, and they can only access a limited amount of cash. The time to set this up is when you're stable—not when you're already hypomanic and every purchase seems brilliant.

The Communication Pause

Hypomania makes you talkative, expansive, and less filtered. This can mean sending emails you'd never send when stable, posting things on social media that damage your reputation, picking fights, making dramatic announcements, or sharing personal information you'll later regret.

The communication pause rule: When you recognize hypomanic symptoms, do not send any important communication that hasn't been reviewed by a trusted person.

Practical steps: - Draft important emails and wait 24 hours before sending - Ask a trusted friend or partner to review any significant communications - Stay off social media or use a "read only" mode - Do not engage in

arguments or difficult conversations - Do not make announcements about life changes (quitting jobs, ending relationships, moving) - Set an auto-responder if needed, or have someone else monitor your email

The communication pause is humbling. It means admitting that you can't trust your own judgment right now. But the women who use it consistently report that it has saved them from career damage, relationship ruptures, and profound public embarrassment.

Sleep Enforcement

This is the most important containment strategy of all. From Chapter 4, you know that sleep disruption can trigger mania. The reverse is also true: mania disrupts sleep, which worsens mania, which further disrupts sleep. This feedback loop is what turns a manageable hypomanic episode into a full-blown manic crisis.

Sleep enforcement means using every tool at your disposal to get adequate sleep, even when your brain is telling you sleep is unnecessary:

- Go to bed earlier than usual (hypomania often delays sleep onset, so start earlier)
- Use prescribed sleep medications or PRN (as-needed) medications as directed
- Eliminate all stimulants (caffeine, nicotine, stimulating activities)
- Use your most effective relaxation techniques
- Ask a partner to help enforce your bedtime

- If you can't sleep, stay in bed with lights off and practice relaxation—rest is still stabilizing
- Do not stay up to "use" the creative or productive energy—it's not a gift, it's a symptom

Some women find that taking a sleep medication (like quetiapine or olanzapine) at the first sign of hypomania can abort the episode entirely. Talk to your psychiatrist about this strategy in advance so you have a plan ready.

The Hypomania Action Plan

Here's what an effective hypomania action plan looks like. Create your own version during a stable period and keep it where you can find it.

When I recognize three or more signs on the hypomania checklist, I will:

1. **Contact my support team.** Text or call my psychiatrist, my partner, and one trusted friend. My message: "I'm noticing hypomanic symptoms. I'm implementing my action plan. I'll check in again in 24 hours."
2. **Freeze my spending.** Hand over credit cards if needed. Set a cash-only limit. 48-hour rule on any purchase > \$50.
3. **Pause all major communications.** Do not send emails, make announcements, or engage in arguments without a review from my trusted person.

4. **Enforce sleep.** In bed by 9 PM. Use sleep medication as prescribed. No exceptions for three nights minimum.
5. **Simplify my schedule.** Cancel all non-essential commitments. No new projects. No social events that aren't explicitly calming.
6. **Track everything.** Increase mood tracking to twice daily. Log sleep, spending, social media use, and any impulsive behaviors.
7. **Reassess in 48 hours.** If symptoms are worsening despite these measures, escalate to my psychiatrist.

What Your Support Team Needs to Know

You can't contain an episode alone. Hypomania and mania are states where your judgment is compromised—that's literally part of the diagnostic criteria. You need people who can see what you can't, who can hold the line when you're pushing against it, who can take the keys (literally and metaphorically) when you're insisting you don't need them.

Your support team might include: - Your psychiatrist or psychiatric nurse practitioner - A therapist who understands bipolar - A partner, spouse, or family member - One or two close friends - A sponsor or peer support contact (if you use support groups)

Here's what they need to know in advance:

Your early warning signs. Your team needs to know what to watch for. Share your mood pattern map with them. "If you notice me sleeping less, starting lots of projects, or talking faster than usual, please tell me. Even if I get defensive."

Your action plan. Give them a written copy of your hypomania action plan. "If I'm in an episode, here's what I need you to help me with: enforcing sleep, pausing spending, and reviewing communications."

What "help" looks like. Be specific. "Help" doesn't mean "take over my life." It means: - "Please check in with me daily and ask directly: 'Are you sleeping? Have you taken your medication? Have you spent money today?'" - "Please help me stick to my bedtime by coming to bed at the same time." - "Please hold my credit cards until I'm stable again." - "Please review emails before I send them, especially to my boss or anyone important." - "Please call my psychiatrist if you're worried and I'm not taking action."

How to talk to you during an episode. This matters enormously. When you're hypomanic, being told to slow down feels like being attacked. You need your team to communicate in ways that you can hear, even in an elevated state.

Effective phrases: - "I see you're feeling amazing right now, and I love that. Can we talk about making sure you stay safe while you enjoy this energy?" - "I'm not saying this is a bad idea. I'm saying let's wait 48 hours and see if it still feels like a good idea." - "I hear how important this

project is to you. Can we make a plan to work on it during the day and protect your sleep at night?" - "I'm worried about you. I love you and I'm worried. Can we call your doctor together?"

Ineffective phrases: - "You're manic, you need to calm down." (Will make you defensive and less likely to listen) - "This is a bad idea." (Too vague, too dismissive of your experience) - "You're scaring me." (Puts their anxiety on you when you can't manage it) - "Just go to sleep." (If you could, you would)

The hard boundaries. Your team also needs to know when to act without your consent. This is uncomfortable to discuss, but it's necessary. At what point should they call your psychiatrist? At what point should they take you to the hospital? What behavior is the absolute line: threats of self-harm, psychosis, dangerous spending, reckless driving?

Have this conversation when you're stable. "If I'm in a manic episode and I'm doing X, please don't ask my permission. Please take action. I will thank you later, even if I'm angry at you in the moment."

The Aftermath

When the episode passes—and it will pass—you'll likely face a period of reckoning. The depression that follows mania can be severe. The shame, guilt, and regret about things you did or said while elevated can be overwhelming.

This is when your support team is most needed. The aftermath of an episode is not the time for self-criticism. It's the time for self-compassion, for reviewing what happened and what you can learn, for repairing relationships, and for returning to your baseline routines.

Some questions to ask yourself after an episode: - What were the early signs I missed? - What containment strategies helped? What didn't? - What would I do differently next time? - What repairs need to be made (financial, relational, professional)? - What did I learn about my patterns that I can use going forward?

Treat each episode as data. Not as a failure. The episode happened because you have a medical condition, not because you're weak, not because you didn't try hard enough. The question is never "how do I prevent every episode forever"—that's not realistic. The question is "how do I make the next episode less severe, shorter, and less damaging than the last one?"

That's progress. That's what this toolkit is for. And you can make that kind of progress, one episode at a time, until the upward spirals become something you recognize, contain, and survive without losing yourself.

Toolkit Summary

- **Use the Hypomania Checklist.** Review it regularly, especially during periods of feeling unusually good. Three or more checked items = time to act.
- **Implement a spending freeze immediately.** Give someone else your credit cards, set a low daily cash limit, and enforce a 48-hour waiting rule on any significant purchase.
- **Pause all major communications.** Do not send important emails, make announcements, or have difficult conversations without a trusted person's review.
- **Enforce sleep aggressively.** Sleep is the most powerful brake on an upward spiral. Use medications, routines, and support people to ensure you get it.
- **Arm your support team in advance.** Give them your early warning signs, action plan, and boundaries for when to act without your consent. Have the hard conversations while you're stable.

Chapter 6: Navigating Depression

When the World Goes Gray

THERE IS A PARTICULAR FLAVOR OF DESPAIR THAT COMES WITH BIPOLAR DEPRESSION THAT IS DIFFICULT TO DESCRIBE TO SOMEONE WHO HAS NEVER EXPERIENCED IT. IT IS NOT SADNESS IN THE WAY MOST PEOPLE UNDERSTAND SADNESS. IT IS A HEAVINESS THAT SETTLES INTO YOUR BONES, A DIMMING OF THE WORLD'S COLOR, A SENSE THAT JOY WAS SOMETHING YOU ONCE READ ABOUT IN A BOOK BUT NEVER TRULY FELT. FOR WOMEN WITH BIPOLAR DISORDER, DEPRESSIVE EPISODES ARE NOT JUST POSSIBLE—THEY ARE, FOR MANY, THE DOMINANT EXPERIENCE. STUDIES SHOW THAT PEOPLE WITH BIPOLAR II DISORDER SPEND ROUGHLY THREE TIMES AS MANY WEEKS IN DEPRESSION AS IN HYPOMANIA, AND EVEN IN BIPOLAR I, DEPRESSIVE EPISODES TEND TO BE LONGER AND MORE FREQUENT THAN MANIC ONES.

This chapter is about navigating those dark waters. Not with platitudes about "thinking positive" or "just getting some exercise"—you have heard enough of those. This is about practical, evidence-based strategies for recognizing, managing, and surviving bipolar depression, with the understanding that depression is a medical event, not a moral failure.

Recognizing Bipolar Depression

One of the most dangerous misconceptions about bipolar depression is that it feels the same as unipolar (major) depression. It does not, and the differences matter enormously for treatment.

Key Differences from Unipolar Depression

Bipolar depression is more likely to involve:

- **Hypersomnia rather than insomnia.** You may sleep twelve to sixteen hours a day and still wake exhausted. The bed feels like a gravitational force you cannot escape.
- **Psychomotor retardation.** Your thoughts feel sluggish. Your body moves slowly. Even lifting your arm to turn a page feels like an effort of will.
- **Weight gain and increased appetite,** particularly cravings for carbohydrates. The body craves energy it cannot seem to generate.
- **Atypical features** such as leaden paralysis—a physical heaviness in the limbs.
- **Mixed features.** This is critical: many women experience depression mixed with agitation, racing thoughts, or irritability. You may feel unbearably sad and wired at the same time, unable to rest but unable to find relief.

- **More frequent episodes.** Women with bipolar tend to experience more rapid cycling and more depressive episodes than men with the condition.

The Seasonal and Hormonal Connection

Bipolar depression often follows patterns that are unique to women. Depressive episodes may cluster around specific phases of the menstrual cycle, particularly the luteal phase (the week or two before your period). For some women, this creates a "double depression" pattern where PMS or PMDD amplifies the bipolar depression, or where the hormonal drop after ovulation triggers a depressive dip.

Winter months can also be treacherous. Seasonal affective patterns are common in bipolar disorder, and light therapy—while effective for unipolar SAD—can actually trigger mania or hypomania in people with bipolar. This is why self-diagnosis and self-treatment of "winter depression" without a psychiatrist's oversight can be dangerous when you have bipolar.

The Medication Trap

Here is the hardest truth in this chapter: when you are depressed, your brain will tell you that the medications are not working. It will whisper that you were fine before you started taking them, that they are making things worse, that what you really need is to stop everything and just reset.

Do not listen to that voice alone.

The urge to stop medications during depression is one of the most common—and most dangerous—patterns in bipolar disorder. Here is why:

1. **Many mood stabilizers and antidepressants take weeks to work.** If you stop during a depressive episode, you may never reach the therapeutic window that would have pulled you out.
2. **Stopping abruptly can trigger a rebound episode—** often a manic or mixed episode that is worse than the original depression.
3. **You lose the protection against mania.** Many people stop their mood stabilizers during depression because "they aren't doing anything for my mood," forgetting that the lithium or lamotrigine is also holding a manic episode at bay.
4. **Kindling.** Each untreated episode makes future episodes easier to trigger and harder to treat. This is called kindling, and it is one of the most compelling reasons to maintain treatment even when you feel like giving up.

This does not mean you should never change your medication. It means you should never change it alone, in the middle of an episode, without your psychiatrist's input. If your current regimen is not working, the answer is to call your doctor, not to flush your pills.

What to Do Instead of Stopping

Create a "medication contract" with yourself—a prewritten agreement, signed when you were stable, that says: "I will not stop or change my psychiatric medications without speaking to my doctor first, even if I think they are not working." Give a copy to your partner, a trusted friend, or your therapist. When depression hits and the voice gets loud, they can remind you what you promised yourself when you were well.

Behavioral Activation: The Antidote to Paralysis

Behavioral activation is one of the most effective non-medication strategies for bipolar depression. The premise is simple but counterintuitive: **behavior change precedes mood change**. You do not wait until you feel motivated to act. You act first, and motivation follows.

The Problem with "Rest Until You Feel Better"

When you are depressed, every instinct tells you to withdraw. Cancel plans. Stay in bed. Wait until you feel stronger. The problem is that withdrawal fuels depression. You lose the small positive reinforcements that come from daily life—the warmth of sunlight, the satisfaction of a completed task, the sense of connection from a brief conversation. Your world shrinks, and depression fills the empty space.

The Behavioral Activation Protocol

Step 1: Track your baseline. For three days, write down what you actually do, hour by hour. Do not judge it. Just observe. You may discover that you are spending seventeen hours a day in bed or on the couch.

Step 2: Identify small, achievable activities. Not "go for a five-mile run." Start with "stand up and stretch for thirty seconds" or "walk to the mailbox." Use the "5-Minute Rule": commit to doing something for just five minutes. If you want to stop after five minutes, you can. Most of the time, you will keep going.

Step 3: Rate the difficulty and the reward. Before each activity, predict how hard it will be (1-10) and how much better you will feel afterward (1-10). Afterward, record what actually happened. You will likely discover that activities are consistently less difficult and more rewarding than your depression predicted. This is called "anticipatory anhedonia"—depression's tendency to drain the imagined pleasure from future events while the actual pleasure, though diminished, still exists.

Step 4: Schedule, do not wait for motivation. Motivation is unreliable when you are depressed. Do not wait for it. Put activities on your calendar and treat them like appointments. Use the "if-then" rule: "If it is 10:00 AM, then I will step outside for two minutes." Remove the decision-making step.

What Counts as Activation?

Almost anything that gets you moving counts:

- Washing your face and brushing your teeth
- Making a cup of tea
- Opening the curtains
- Sending a one-sentence text to a friend
- Standing on the porch for thirty seconds of fresh air
- Petting a cat or dog for two minutes
- Listening to one song all the way through
- Eating something nutritious (even if you are not hungry)

The goal is not to cure the depression in a day. The goal is to interrupt the downward spiral of withdrawal and inactivity, one tiny act at a time.

The Depression Toolkit

Every woman with bipolar needs a personalized depression toolkit—a concrete list of what helps and what hurts, written during a stable period and kept somewhere accessible.

What Helps

Structure and routine. When everything feels impossible, routine is a raft. Go to bed at the same time. Eat meals at roughly the same time. Even if you cannot do much, doing it on a schedule provides a grounding structure.

Light exposure. Morning light (natural sunlight or a light box) can help regulate circadian rhythms. But be cautious: if you have bipolar I, talk to your doctor before using a light box, as it can trigger mania.

Social connection (on your terms). Isolation makes depression worse, but large social demands can be overwhelming. Find the middle ground: a text exchange, a ten-minute phone call, a quiet coffee with one trusted friend.

Small physical movement. A gentle walk, stretching, or yoga. The goal is not fitness; the goal is breaking the cycle of physical inertia.

Cognitive defusion. This is a technique from ACT (Acceptance and Commitment Therapy). Instead of fighting depressive thoughts, you simply observe them: "I notice that I am having the thought that I am worthless." This creates distance between you and the thought. It loses some of its power.

One thing done. Completing even one small task—washing a single dish, folding one item of laundry—releases a small amount of dopamine. The goal is not to clean the house. The goal is to prove to your brain that action is possible.

Professional support. Therapy, medication adjustments, and if needed, more intensive interventions like TMS (transcranial magnetic stimulation) or ECT (electroconvulsive therapy). These are not last resorts. They are tools.

What Hurts

Alcohol and recreational drugs. These destabilize mood and interfere with medications. Alcohol is a central nervous system depressant—it may provide temporary relief but deepens the depression overall.

Major life decisions. Never quit a job, end a relationship, or move during a depressive episode. Your brain is not capable of accurate risk assessment. Make a rule: no major decisions until you have been stable for at least two weeks.

Social isolation. This is a tricky one because isolation feels protective. But more than a day or two of total withdrawal is a warning sign.

Sleep disruption. Both too much and too little sleep worsen depression. Protect your sleep schedule like your life depends on it, because it does.

Caffeine overuse. The temporary lift is followed by a crash that deepens depression. Limit caffeine to the morning hours.

Self-blame. You are not weak. You are not lazy. You have an illness that attacks the very systems that generate motivation and pleasure. Treating yourself with contempt for being depressed is like blaming a broken leg for not being able to run a marathon.

Safety Planning Basics

Suicide risk in bipolar disorder is tragically high—higher than in almost any other psychiatric condition. Women with bipolar attempt suicide at higher rates than men with bipolar, though men complete suicide at higher rates. **If you are reading this and having thoughts of harming yourself, please know that this is the illness talking, not the truth. These feelings are temporary, even when they feel permanent.**

Creating a Safety Plan

A safety plan is different from a crisis plan (which we will cover in Chapter 11). A safety plan is specifically for suicidal thoughts. It should be written when you are stable and kept where you can find it when you are not.

Step 1: Warning signs. List the specific thoughts, feelings, or behaviors that signal you are entering dangerous territory. "I start thinking my family would be better off without me." "I start researching methods online." "I stop responding to texts."

Step 2: Internal coping strategies. What can you do on your own to manage the thoughts? "Take three slow breaths." "Remind myself that this feeling has passed before." "Listen to my safety playlist." "Hold an ice cube."

Step 3: People and social settings that provide distraction. Who can you call or be with when the thoughts get loud? List specific names and phone numbers. "My sister Sarah: 555-0123." "My friend Jenna: 555-0456."

Step 4: Professional resources. List your psychiatrist, therapist, and the nearest crisis line. The 988 Suicide and Crisis Lifeline (US) is available 24/7. Write this number down. Put it in your phone. Put it on your bathroom mirror.

Step 5: Making the environment safe. Identify ways to reduce access to means. This might mean asking a trusted person to hold your medications and dispense them daily, removing firearms from the home, or not keeping an excess supply of prescription pills.

Step 6: The emergency plan. If none of the above steps work, the plan is clear: go to the nearest emergency room or call 911. Do not drive yourself. Do not wait to "see if it passes." The emergency room is not a sign of failure. It is a sign that you are using your tools.

Moving Forward

Navigating depression with bipolar disorder is not about never feeling depressed again. It is about recognizing depression for what it is—a phase, an episode, a weather pattern in the brain—and responding with strategy rather than surrender. The depression will pass. It always has, and it always will. Your job is to survive it intact, with your relationships, your job, and your life as intact as possible.

Toolkit Summary

- **Bipolar depression differs from unipolar depression** — look for hypersomnia, psychomotor retardation, weight gain, mixed features, and atypical symptoms
- **Never stop or change medications without talking to your doctor** — create a medication contract during stable times

- **Use behavioral activation** — act first, motivation follows; start with five-minute activities and schedule them like appointments
- **Build your depression toolkit** — know what helps (light, routine, movement, connection) and what hurts (alcohol, isolation, sleep disruption, major decisions)
- **Create a written safety plan** — warning signs, coping strategies, contacts, environmental safety, and emergency steps
- **Depression is a medical event, not a moral failure** — treat yourself with the same compassion you would offer a friend with a serious illness

Chapter 7: Relationships and Boundaries

The Relationship Paradox

BIPOLAR DISORDER IS NOT A SOLITARY ILLNESS. IT RADIATES OUTWARD, TOUCHING EVERYONE YOU LOVE—YOUR PARTNER, YOUR FAMILY, YOUR CLOSEST FRIENDS. THE VERY PEOPLE WHO BECOME YOUR LIFELINE DURING DIFFICULT EPISODES ARE OFTEN THE SAME PEOPLE WHO ABSORB THE MOST COLLATERAL DAMAGE. THIS CREATES WHAT I CALL THE RELATIONSHIP PARADOX OF BIPOLAR: YOU NEED YOUR RELATIONSHIPS TO SURVIVE THE ILLNESS, BUT THE ILLNESS CAN MAKE RELATIONSHIPS FEEL IMPOSSIBLE TO MAINTAIN.

This chapter is about navigating that paradox. Not by pretending bipolar does not affect your relationships—it does, and pretending otherwise only creates more pain—but by developing concrete strategies for honesty, boundary-setting, communication, and mutual support. The goal is not a perfect relationship. The goal is a resilient one.

How Bipolar Affects Relationships

Understanding the patterns is the first step to managing them. Bipolar disorder tends to strain relationships in predictable ways.

During Depression

When you are depressed, relationships often suffer from withdrawal. You cancel plans. You stop answering texts. You may become irritable or tearful without clear cause. Your partner may feel rejected, confused, or helpless. They want to help but do not know how, and your withdrawal signals that their efforts are unwanted.

The danger here is that your partner begins to interpret your illness-related behavior as a personal statement. "She does not want to spend time with me." "She does not love me anymore." Without clear communication about what depression is and what it is not, the relationship develops small fractures that accumulate over time.

During (Hypo)mania

When you are hypomanic, the relationship stressors are different but equally damaging. You may be irritable, argumentative, or hypersexual. You may make impulsive decisions that affect both of you—spending money, starting projects, making dramatic plans. You may feel "amazing" and have no patience for your partner's slower, more cautious energy.

Paradoxically, some partners find the hypomanic phase more destabilizing than the depression. At least during depression, you are predictably unavailable. During

hypomania, you are unpredictably intense—sometimes euphoric and expansive, sometimes raging and accusatory.

During Stability

Even during stable periods, the shadow of past episodes lingers. Your partner may be hypervigilant, watching for signs of another episode. They may have their own trauma from past crises—the hospitalizations, the frantic phone calls, the financial messes. Trust, once broken by the illness, is slow to rebuild.

This is why relationship work cannot wait until you are in crisis. The foundation for a strong relationship during difficult times is built during the good times.

When to Tell Partners: The Disclosure Decision

One of the most anxiety-provoking questions for women with bipolar is: when do I tell a new partner?

There is no single right answer, but there are frameworks to help you decide.

The General Guidelines

- **Do not tell on the first date.** You are not obligated to disclose your medical history to a stranger. The first few dates are for assessing basic compatibility and safety.

- **Consider telling before things become serious.** If you see potential for a long-term relationship, disclosure should happen before you are deeply emotionally invested. This gives both of you the opportunity to make informed decisions.
- **Tell when you are stable.** Never disclose during a mood episode. You want your partner to meet the real you, not the depressed or hypomanic version.
- **Frame it as information, not an apology.** You do not need to ask for permission to have bipolar disorder. You are sharing information about yourself. The tone should be matter-of-fact: "This is part of my life, and here is what it means for a relationship."

The Disclosure Script

Here is a template for how to tell a new partner. Adjust the language to fit your voice:

"I want to share something important about my health because I care about you and I want our relationship to be built on honesty. I have bipolar disorder. For me, this means that I experience periods of depression and periods of elevated mood that are different from normal ups and downs. I manage it with medication, therapy, and lifestyle routines, and I have been stable for [X period]. I

do not expect it to define our relationship, but it is part of my life, and I wanted you to know so you can understand me better. Do you have any questions?"

After sharing, give them space to process. They may need time. They may have questions. They may need to do their own research. That is all normal.

What If They React Poorly?

Some people will react badly. They may say insensitive things, or they may end the relationship. This hurts, but it is also information—information that this person was not equipped to be the partner you need. Bipolar disorder is a significant condition, and not everyone has the emotional resources or understanding to be a good partner to someone who lives with it. It is better to know this early than to invest years in a relationship that will crumble under pressure.

Communication Scripts for Different Moods

One of the most powerful tools you can develop is the ability to communicate about your mood state directly, without expecting your partner to read your mind.

When You Are Depressed

"I am in a depressive episode right now. This is not about you. I need some space, but I also need to know you are there. Can we check in briefly at the end of the day?"

Here is what I can handle right now: [light conversation / movies together / just being in the same room without talking]."

When You Are Hypomanic

"I am feeling elevated right now, and I know my judgment may not be reliable. I would like your help with impulse control. Can I run this decision by you before I act on it? If I seem intense or irritable, please let me know gently."

When You Need Help

"I think I might need more support than you can give me right now. Could you help me contact my doctor? Or would you be willing to come with me to my appointment?"

When They Ask "Are You Okay?"

This question can feel loaded, especially when you are not okay. If you are not okay but not in crisis, try: "I am struggling, but I am safe. I am using my tools. I will let you know if that changes."

When You Have Hurt Them

"I know my behavior [during the episode / when I was irritable] hurt you. That was not who I am—that was the illness. I am sorry for the impact it had on you. Can we talk about what you need from me to rebuild trust?"

Boundary Setting During Stable Times

Boundaries are not walls. They are agreements that protect both people in a relationship. For women with bipolar, clear boundaries are not optional—they are essential for stability.

Boundaries Around Treatment

- "I will take my medications as prescribed, and I expect you to support that without questioning whether I really need them."
- "If I say I need to see my psychiatrist, I need you to trust that I know my own warning signs."
- "I will not tolerate being shamed for having a mental illness."

Boundaries Around Caretaking

- "You are allowed to take care of yourself, even if that means stepping back when I am struggling."
- "I will not expect you to be my only source of support. I maintain a team: doctor, therapist, support group."
- "If I am in crisis and refusing help, you have my permission to call my doctor or emergency services, even if I am angry about it in the moment."

Boundaries Around Behavior

- "It is not acceptable to yell at me, blame me for my illness, or threaten to leave during an episode."
- "It is acceptable to say, 'I need a break from this conversation right now,' and walk away."
- "If I am hypomanic and making reckless decisions, you have my permission to say no."

The Boundary Conversation

Set aside a specific time to discuss boundaries—not during an argument, not during an episode. Use this script as a starting point:

"I want to talk about how we handle my bipolar together, so we both feel safe and supported. I have some ideas about what I need from you, and I would like to hear what you need from me. Can we set aside an hour this weekend to talk about this?"

Building a Support Network

Your partner cannot be everything. No single person can carry the weight of supporting someone through bipolar episodes. Building a broader support network is not just good for you—it is also essential for protecting your relationship.

Who Should Be in Your Network

- **Your treatment team:** psychiatrist, therapist, primary care provider
- **Trusted family members:** parents, siblings, adult children who understand your condition
- **Friends who "get it":** people you can call without having to explain everything from scratch
- **Peer support:** a DBSA or NAMI support group, or an online community like BDDI (Bipolar Disorder Depression and Irritability support)
- **Crisis resources:** your local crisis line, 988, a crisis response team

Creating a Support Map

Draw a circle. Put your name in the middle. Around it, place the names of people who support you, organized by how close they are to you. The innermost ring is your emergency contacts—people who would drop everything if you called. The next ring is your daily support—people you check in with regularly. The outer ring is your community—support group members, online friends, acquaintances who understand.

Your partner does not need to be in every ring. In fact, it is healthier if they are not. Your partner should know who is in your inner rings so that if you are not able to ask for help, they can.

Supporting Your Partner

Your partner needs support too. Caregiver burnout is real, and it is one of the leading causes of relationship breakdown in bipolar couples.

What Your Partner Might Be Experiencing

- **Grief for the relationship they imagined** — the one without hospital visits, medication schedules, and mood tracking
- **Hypervigilance** — constantly watching for signs of an episode, never fully relaxing
- **Isolation** — not feeling able to talk to friends or family about what is really happening
- **Resentment** — an uncomfortable but real emotion when the illness repeatedly disrupts their plans, their sleep, their life
- **Guilt** — for feeling resentful, for wanting to leave, for not being "supportive enough"

How to Support Your Partner

1. **Encourage them to have their own support system.** Suggest NAMI Family-to-Family classes or a DBSA support group for family members. Give them permission to talk about their experience without feeling disloyal.

2. **Acknowledge their experience.** Say: "I know this is hard for you too. Thank you for being here." Do not assume they know you appreciate them.
3. **Make time for non-illness connection.** Have conversations that are not about bipolar. Go on dates. Laugh together. The relationship needs to be more than the diagnosis.
4. **Have a plan for episodes so they do not have to improvise.** Create a written plan that includes: what signs to watch for, who to call, what medications you take, and what hospital you prefer. This reduces their burden during a crisis.
5. **Check in with them.** Ask: "How are you doing with all of this? What do you need?"

When Professional Help Is Needed

If your relationship is struggling despite your best efforts, consider couple's therapy with a therapist who understands bipolar disorder. This is not a sign of failure. Bipolar disorder is hard on relationships, and professional guidance can make the difference between a relationship that fractures and one that grows stronger.

Toolkit Summary

- **Bipolar affects relationships differently in each mood state** — depression causes withdrawal, hypomania causes intensity, and stability requires rebuilding trust
- **Disclose thoughtfully and from stability** — use a prepared script, give your partner space to process, and accept that not everyone will be equipped for the journey
- **Use direct communication scripts** — tell your partner what mood state you are in, what you need, and what you can handle
- **Set boundaries during stable times** — around treatment, caretaking roles, and acceptable behavior from both sides
- **Build a support network beyond your partner** — no single person can carry the full weight
- **Support your partner too** — acknowledge their experience, encourage their own support system, and make space for non-illness connection

Chapter 8: Career and Financial Stability

The Work-Mood Tightrope

WORK IS COMPLICATED WHEN YOU HAVE BIPOLAR DISORDER. ON ONE HAND, MEANINGFUL WORK PROVIDES STRUCTURE, PURPOSE, SOCIAL CONNECTION, AND FINANCIAL INDEPENDENCE—ALL OF WHICH ARE PROTECTIVE FOR MOOD STABILITY. ON THE OTHER HAND, THE DEMANDS OF MOST WORKPLACES—EARLY MORNINGS, HIGH STRESS, INTERPERSONAL CONFLICT, IRREGULAR SCHEDULES—CAN BE DESTABILIZING. AND THEN THERE IS THE FINANCIAL REALITY: BIPOLAR DISORDER, PARTICULARLY DURING MANIC OR HYPOMANIC EPISODES, CAN DEVASTATE SAVINGS, CREDIT, AND FINANCIAL SECURITY IN A MATTER OF WEEKS.

This chapter is about navigating both of these domains with strategy rather than hope. You cannot predict every mood episode, but you can build systems that protect your career and your finances even when your brain is not cooperating.

Managing Work with Bipolar

The Reality of Work Disruption

Let us be honest about what bipolar disorder does to a career. Research consistently shows that bipolar disorder is associated with higher rates of unemployment, underemployment, and disability leave. The average person with bipolar misses between three and six weeks of work per year due to mood episodes. Even when present, cognitive symptoms—difficulty concentrating, slowed processing, memory lapses—can significantly impair performance.

This is not your fault. It is not a character flaw. It is a medical condition that affects the brain's executive functioning. But acknowledging the reality allows you to plan around it.

Disclosure: To Tell or Not to Tell Your Employer

Disclosure at work carries different risks and benefits than disclosure in relationships. Here is a framework for thinking about it.

Reasons to disclose (to HR, not necessarily your manager):

- It makes you eligible for workplace accommodations
- It protects you from being fired for symptoms (if you are in a jurisdiction with employment protections)
- It can reduce the stress of hiding or lying about appointments

Reasons not to disclose: - Stigma is real, and some managers will unconsciously (or consciously) discriminate
- Once disclosed, you cannot un-disclose - If you do not need accommodations, disclosure may not be necessary

A middle path: Disclose only what is necessary. You do not need to say "bipolar disorder." You can say "a chronic medical condition that requires periodic medical appointments" or "I have a condition that affects my sleep schedule." Request accommodations through HR with a doctor's note that does not specify your diagnosis unless required by law.

Workplace Accommodations That Help

Under the Americans with Disabilities Act (ADA) in the US, bipolar disorder is considered a disability, and employers are required to provide "reasonable accommodations" that do not cause "undue hardship." Here are accommodations that women with bipolar commonly find helpful:

- **Flexible scheduling.** Starting later in the morning (if mornings are difficult due to medication or sleep disruption) or adjusting hours around medical appointments.
- **Remote work or hybrid schedule.** For some women, having the option to work from home during low-grade episodes can prevent a full relapse.

- **Reduced distractions.** A private office or quieter workspace if noise and stimulation trigger irritability or anxiety.
- **Breaks as needed.** The ability to take short breaks for grounding exercises, medication, or simply stepping away from stress.
- **Deadline flexibility.** When processing speed slows during depression, having the ability to extend non-critical deadlines can make the difference between staying afloat and crashing.
- **Supervision adjustments.** Regular check-ins with a supportive supervisor, or written instructions for complex tasks to compensate for memory issues.
- **Leave for treatment.** The ability to use sick leave or FMLA for psychiatric appointments, therapy, or hospitalization.

How to Request Accommodations

1. **Document your needs.** Work with your psychiatrist or therapist to write a letter specifying what accommodations would help you perform your job.
2. **Contact HR.** Most companies have a formal accommodation request process. Submit your doctor's letter and your specific requests in writing.

3. **Frame it as performance-focused.** "These accommodations will help me maintain consistent productivity and attendance." Not: "These accommodations will help me manage my mental illness."
4. **Be prepared for negotiation.** Your employer may offer alternatives. Be willing to discuss, but know your non-negotiables.

FMLA and Disability Rights (US Context)

The Family and Medical Leave Act (FMLA)

FMLA provides up to 12 weeks of unpaid, job-protected leave per year for serious health conditions—including bipolar disorder. Key points:

- **Eligibility:** You must have worked for your employer for at least 12 months (not necessarily consecutive) and at least 1,250 hours in the past 12 months. Your employer must have 50+ employees within 75 miles.
- **Intermittent leave:** You do not have to take all 12 weeks at once. You can use FMLA intermittently—for example, one day per week for therapy appointments, or three days during a depressive episode.
- **Job protection:** Your employer must restore you to your original job or an equivalent position when you return.

- **Medical certification:** Your doctor will need to complete FMLA paperwork certifying that you have a serious health condition.

Short-Term and Long-Term Disability

If FMLA leave is unpaid, short-term disability insurance (if you have it through your employer) can provide partial income replacement during leave. Long-term disability is available if your condition prevents you from working for an extended period.

Important: Bipolar disorder is one of the most commonly approved conditions for long-term disability benefits. If you are unable to work due to bipolar, applying for disability is a reasonable option, not a failure.

What to Do If You Are Denied

If your accommodation request or FMLA leave is denied, or if you experience discrimination, contact:

- **Job Accommodation Network (JAN):**
1-800-526-7234 — free, confidential guidance on workplace accommodations
- **Equal Employment Opportunity Commission (EEOC):** 1-800-669-4000 — files discrimination charges
- **National Alliance on Mental Illness (NAMI):**
nami.org — legal advocacy resources

- **Disability Rights Legal Center:** Your state's Bar Association can refer you to disability rights attorneys

The Spending Freeze During Hypomania

This section may be the most financially important one you read in this book.

During hypomania, the brain experiences a specific change in reward processing. Decision-making becomes impulsive. Risk assessment is impaired. The future feels infinite and bright, and consequences feel unreal. This combination—elevated mood, impaired judgment, reduced impulse control—creates a perfect storm for financial disaster.

The Hypomanic Spending Pattern

It typically follows a recognizable arc: 1. **The exciting idea.** A new project, investment, business venture, or purchase feels brilliant and urgent. 2. **The rationalization.** "I deserve this." "This is an investment in myself." "I can make the money back easily." 3. **The escalation.** One purchase leads to another. Credit cards come out. Savings accounts are tapped. 4. **The aftermath.** The hypomania subsides, and you are left with debt, regret, and sometimes irreparable damage to your financial life.

The Spending Freeze Protocol

This is a pre-committed agreement that you implement the moment you or your trusted person suspects hypomania:

1. **Freeze all credit cards.** Literally. Put them in a glass of water in the freezer, or give them to a trusted person.
2. **Lock your credit.** Freeze your credit reports with all three bureaus (Equifax, Experian, TransUnion). This prevents opening new lines of credit.
3. **Set a spending threshold.** Agree with a trusted person that any non-essential purchase over a small amount (say, \$25 or \$50) requires their approval.
4. **Remove saved payment methods.** Log out of Amazon, Etsy, and any other online shopping accounts. Delete saved credit card information.
5. **Delay major decisions.** Any financial decision that cannot wait two weeks is probably hypomanic. Two-week rule: do not make any purchase or financial commitment over \$100 without waiting 14 days.
6. **Tell someone.** Notify your partner, family member, or sponsor that you are feeling elevated and need help with financial boundaries.

Financial Safeguards for Stable Times

You cannot always trust your future self to make good decisions. The solution is to build safeguards during stable periods that protect you during unstable ones.

Structural Safeguards

- **Separate accounts.** Maintain a savings account that is not linked to your checking account and does not have a debit card. Set up automatic transfers so money goes in but is hard to take out.
- **Low credit card limits.** Request low credit limits on your cards—\$1,000 or \$2,000 maximum. You can always increase them later, but you cannot un-spend money you never had access to.
- **A trusted financial partner.** Give someone you trust—a parent, partner, or close friend—visibility into your accounts. This is not about control; it is about accountability. Some women give a trusted person the ability to freeze access during episodes.
- **Automatic bill pay.** Set all essential bills (rent, utilities, insurance, medication) on autopay from an account you cannot easily spend from.

- **Emergency fund.** Aim for three to six months of essential expenses in a separate account. This is your "episode fund"—money for the times when work is disrupted or impulsive spending happens despite your best efforts.
- **Financial power of attorney.** For women with severe bipolar I, it may be worth discussing a financial power of attorney with a lawyer—a legal document that gives a trusted person authority to manage your finances during incapacitating episodes.

The Post-Episode Financial Recovery Plan

When an episode ends and you are facing the financial fallout, do not panic. You will recover. Here is the process:

1. **Assess the damage honestly.** List all debts, all missed payments, all financial obligations. Do not avoid the numbers.
2. **Do not make it worse.** Do not take out more debt to cover the damage. Do not hide it from your partner.
3. **Prioritize.** Rent/mortgage, utilities, food, medication—these come first. Credit card debt is painful but not immediately dangerous.
4. **Negotiate.** Call credit card companies and explain you experienced a medical crisis. Some will waive late fees or set up payment plans.

5. **Rebuild slowly.** One month at a time. Celebrate small wins: paying off one card, rebuilding savings by \$100.
6. **Forgive yourself.** Financial mistakes during an episode are symptoms of an illness, not moral failures. You made those decisions in an altered mental state. The shame is a burden you do not need to carry.

Career Planning with Mood Cycles in Mind

Traditional career advice assumes a linear path: advance, earn more, take on more responsibility, work harder. For women with bipolar, the optimal career path may look different.

What to Look For in a Career

- **Predictable hours.** Shift work, unpredictable overtime, and frequent travel are destabilizing.
- **Low interpersonal stress.** High-conflict environments are exhausting for anyone, but particularly destabilizing for bipolar.
- **Accommodation-friendly culture.** Some industries and companies are more supportive of mental health needs than others.
- **Good benefits.** Health insurance with good mental health coverage is not a perk—it is a necessity.

- **Flexibility.** The ability to adjust hours or work location during episodes is invaluable.

What to Avoid

- **Commission-only or high-pressure sales.** The income instability adds financial stress, and the high-stakes environment can trigger episodes.
- **Entrepreneurship during unstable periods.** Starting a business requires exactly the kind of sustained executive function, consistent energy, and risk tolerance that bipolar disrupts during episodes. This is not to say women with bipolar cannot be entrepreneurs—many are brilliant ones—but attempt it from stability, not from hypomanic inspiration.
- **Careers that require perfect attendance.** Nursing, teaching, law—professions where missing work has severe consequences—can be done, but you need explicit accommodations and backup plans.

The Career Map

Think of your career not as a ladder but as a series of terraces. You move up when stable, and you hold steady during episodes. Build skills during your good periods that make you valuable enough to retain during your difficult periods. And always, always keep an updated

resume and professional network—not because you expect to leave, but because having options reduces the terror of a potential job loss.

Toolkit Summary

- **Workplace accommodations are your right** — request flexible scheduling, remote work, breaks, and deadline flexibility through HR with a doctor's letter
- **FMLA provides job-protected unpaid leave for bipolar** — you can use it intermittently for appointments or episodes
- **Implement a spending freeze at the first sign of hypomania** — freeze cards, lock credit, set a low spending threshold with a trusted person
- **Build structural financial safeguards during stable times** — separate accounts, low credit limits, automatic bill pay, and an emergency fund
- **Plan your career with mood cycles in mind** — prioritize predictability, flexibility, and good benefits over income maximization
- **Financial mistakes during episodes are symptoms, not moral failures** — assess damage honestly, prioritize essentials, negotiate, rebuild slowly

Chapter 9: Hormones, Pregnancy, and Menopause

The Female Factor

FOR DECADES, BIPOLAR DISORDER WAS STUDIED PRIMARILY IN MEN. THE RESEARCH ON HOW FEMALE REPRODUCTIVE HORMONES INTERACT WITH MOOD DISORDERS IS STILL CATCHING UP, BUT WHAT WE KNOW IS CLEAR: ESTROGEN, PROGESTERONE, AND OTHER REPRODUCTIVE HORMONES DIRECTLY AFFECT BRAIN CHEMISTRY, AND FOR WOMEN WITH BIPOLAR, THESE FLUCTUATIONS CAN BE PROFOUNDLY DESTABILIZING.

This is not just another thing to worry about. It is information—and information is power. Understanding the intersections between your reproductive cycle and your bipolar disorder allows you to anticipate, plan, and advocate for the care you need at every stage of life.

How the Menstrual Cycle Affects Mood

The Hormonal Landscape

Your menstrual cycle is divided into two main phases:

- **Follicular phase (day 1 to ovulation):**

Estrogen rises gradually, peaking just before ovulation. This is generally a more stable or even mood-elevating phase for many women.

- **Luteal phase (ovulation to menstruation):**

After ovulation, progesterone rises and estrogen drops, then both hormones fall sharply just before menstruation.

For women with bipolar, the luteal phase is especially significant. The drop in estrogen affects serotonin, dopamine, and GABA receptors—all of which are already vulnerable in bipolar brains. The result is that many women experience a predictable worsening of mood symptoms in the week or two before their period.

Tracking the Pattern

The first step is to discover whether you have a menstrual mood pattern. Track two variables simultaneously:

1. Your daily mood (using an app like eMoods or Daylio)
2. Your menstrual cycle phase (using a period tracking app or calendar)

After three to six months, look for patterns. Do you consistently dip in the week before your period? Do your depressive episodes start at a specific point in your cycle? Do you experience breakthrough hypomania during ovulation?

This information is gold. Share it with your psychiatrist. Some women benefit from small medication adjustments during the luteal phase—for example, a temporary increase in a mood stabilizer or the addition of a short-term medication to bridge the hormonal drop.

PMDD and Bipolar: A Complicated Overlap

Premenstrual Dysphoric Disorder (PMDD) affects approximately 3-8% of women, but rates are higher among women with bipolar disorder. PMDD is characterized by severe mood symptoms—depression, irritability, anxiety—that appear in the luteal phase and resolve with menstruation.

If you suspect PMDD on top of bipolar, this requires specialized care. Both conditions need to be treated, and some treatments for PMDD (like SSRIs) can destabilize bipolar mood. A reproductive psychiatrist—a psychiatrist who specializes in the intersection of mental health and female reproductive hormones—is the ideal provider.

Pre-Conception Planning

If you are reading this section, you may be thinking about pregnancy. You may be terrified that bipolar disorder makes pregnancy impossible, or dangerous, or selfish. Let me say this clearly: **Many women with bipolar disorder have healthy pregnancies and healthy babies. The key is planning, not avoidance.**

The Pre-Conception Conversation

At least six months before you start trying to conceive, schedule an appointment with both your psychiatrist and your OB-GYN. Ideally, these providers should talk to each other. The agenda:

1. **Medication review.** Which medications are safe during pregnancy? Which need to be changed or stopped? Which can be continued with careful monitoring?
2. **Risk assessment.** What is your relapse risk if you stop or change medications? What is the fetal risk of each medication? These risks need to be compared honestly, not assumed.
3. **Folic acid supplementation.** All women planning pregnancy should take folic acid. Women on valproate (Depakote) or carbamazepine (Tegretol) need higher doses, as these medications interfere with folate metabolism.

4. **Genetic counseling.** If you or your partner carry genetic risk factors, a genetic counselor can help you understand the heritability of bipolar disorder (which is real but not deterministic—most children of parents with bipolar do not develop the condition).

The Medication Discussion

This is often the most anxiety-provoking part. Let me offer a framework:

The risk of taking medication during pregnancy must be weighed against **the risk of stopping medication and having a mood episode during pregnancy.**

Untreated mood episodes during pregnancy carry real risks: - Poor prenatal care (depression makes it hard to attend appointments, eat well, or follow medical advice) - Increased risk of preterm birth, low birth weight, and pregnancy complications - Postpartum relapse (the best predictor of postpartum episode is antenatal episode) - In severe cases, suicidal ideation or psychotic symptoms that endanger both mother and baby

The most dangerous time for medication changes is during the first trimester, when fetal organs are forming. But some medications are safer than others. In general: - **Lithium:** Associated with a small risk of cardiac malformation (Ebstein's anomaly), but the absolute risk is low (0.05-0.1%). With careful monitoring

and dose adjustment, many women take lithium through pregnancy. - **Lamotrigine:** Generally considered one of the safer mood stabilizers during pregnancy, though doses may need to be increased as pregnancy progresses. - **Quetiapine and other atypical antipsychotics:** Widely used in pregnancy with reasonable safety data. Weight gain and metabolic effects need monitoring. - **Valproate (Depakote):** Generally not recommended during pregnancy due to higher risks of neural tube defects and neurodevelopmental effects. - **Carbamazepine:** Associated with neural tube defects; generally avoided.

There is no perfect answer. Every woman—with her doctors—must make the decision that balances her unique risks. Do not let anyone shame you for the decision you make. If you choose to continue medication, you are not a bad mother. If you choose to discontinue, you are not reckless. You are making an impossible choice with imperfect information.

Pregnancy and Bipolar

Mood Trajectory During Pregnancy

Conventional wisdom once held that pregnancy was protective against mood episodes (thanks to high estrogen levels). We now know this is not universally true. While some women do experience mood stability during pregnancy, others experience worsening:

- **Approximately 20-30% of women with bipolar will experience a mood episode during pregnancy** if not on medication.
- **Discontinuing mood stabilizers before pregnancy carries a high relapse risk**—some studies suggest 70-80% relapse within months of stopping.
- **Relapses during pregnancy tend to be depressive** or mixed, though manic episodes do occur.

Prenatal Care Adaptations

You will need more monitoring than a woman without bipolar. This might include: - More frequent psychiatric appointments - More frequent OB-GYN visits - Fetal monitoring if you are on lithium (lithium levels need to be checked monthly, and a fetal echocardiogram at 18-20 weeks is recommended) - Collaboration between your psychiatrist and OB-GYN—they need to talk to each other, not just both talk to you

The Birth Plan

Include mental health in your birth plan: - Who will manage your medication during labor? - How will sleep disruption be minimized after birth? - What is the plan if you become manic or psychotic postpartum? - Who will watch for symptoms if you cannot?

Consider writing a separate "postpartum mental health plan" that covers the first eight weeks after birth.

Postpartum Mood Risk

The Dangerous Window

The postpartum period is the highest-risk time in the life of a woman with bipolar disorder. The risk of a severe mood episode in the first three months after giving birth is **staggering**:

- **30-50% of women with bipolar will experience a postpartum episode** if not on prophylactic medication
- **The risk of postpartum psychosis is 100-500 times higher** in women with bipolar than in the general population
- **Postpartum psychosis is a medical emergency** requiring immediate hospitalization

This is not meant to terrify you. It is meant to prepare you. The postpartum period is a known high-risk window, and known risks can be managed.

Postpartum Psychosis: Know the Signs

Postpartum psychosis typically begins within the first two weeks after birth (though it can occur up to three months later). Symptoms include: - Rapidly changing mood (euphoria to despair within hours) - Confusion, disorientation, or bizarre behavior - Delusions (often about the baby—that the baby is special, possessed, or in danger) - Hallucinations - Severe insomnia when sleep is possible - Rapid speech, agitation, paranoia

Postpartum psychosis always requires emergency treatment. If you or your partner notice these signs, go to the emergency room immediately. This is not something to "wait and see" about.

The Postpartum Plan

The plan should include: 1. **Medication.** Most women are advised to restart or continue mood stabilizers immediately after birth. 2. **Sleep.** Sleep deprivation is the number one trigger for postpartum mania. Designate someone (partner, family member, friend) to take over night feedings so you can get at least four to five hours of uninterrupted sleep. 3. **Support schedule.** Arrange for help for the first 4-8 weeks: meals, household tasks, baby care. 4. **Monitoring.** Your partner or family member should have a list of warning signs and instructions for what to do if they appear. 5. **Therapy.** Postpartum therapy with a therapist who understands bipolar is not

optional. 6. **Emergency plan.** Who will you call? Where will you go? Who will care for the baby if you need hospitalization?

Breastfeeding and Medication

Breastfeeding decisions are deeply personal. Many psychiatric medications pass into breast milk in small amounts. The risk to the infant is generally low for most mood stabilizers (lamotrigine, quetiapine, olanzapine), but lithium requires careful monitoring of the infant's lithium levels.

The key question is not "Is it safe to breastfeed on this medication?" but "Is the risk of formula-feeding (loss of sleep, loss of convenience, personal cost) greater or lesser than the minimal medication exposure?" There is no universally correct answer. Make this decision with your psychiatrist and your baby's pediatrician.

Perimenopause and Bipolar

The Second High-Risk Window

Perimenopause—the years-long transition before menstruation stops—is another period of hormonal chaos. Estrogen no longer rises and falls in predictable cycles. Instead, it fluctuates wildly: some months very high, others very low. Progesterone declines more steadily.

For women with bipolar, this hormonal instability can trigger mood destabilization even in women who have been stable for years. Common patterns include: - More frequent mood episodes - More severe episodes - New onset of rapid cycling - First onset of mixed episodes - Worsening of depression (estrogen's mood-protective effects decline) - Increased sensitivity to sleep disruption

Strategies for Perimenopause

1. **TRACK EVERYTHING.** MOOD, MENSTRUAL CYCLE, SLEEP, AND PERIMENOPAUSAL SYMPTOMS (HOT FLASHES, NIGHT SWEATS, SLEEP DISRUPTION). THIS DATA WILL HELP YOU AND YOUR DOCTORS SEE PATTERNS.
2. **CONSIDER HRT.** HORMONE REPLACEMENT THERAPY (HRT) CAN STABILIZE ESTROGEN FLUCTUATIONS AND MAY IMPROVE MOOD STABILITY FOR SOME WOMEN WITH BIPOLAR. THE RESEARCH IS LIMITED, BUT SOME WOMEN FIND THAT LOW-DOSE ESTROGEN PATCHES SMOOTH OUT THE HORMONAL ROLLER COASTER. WORK WITH BOTH YOUR PSYCHIATRIST AND YOUR OB-GYN.
3. **ADJUST MEDICATIONS.** MANY WOMEN NEED MEDICATION ADJUSTMENTS DURING PERIMENOPAUSE. WHAT WORKED FOR THE PAST FIVE YEARS MAY STOP WORKING. THIS IS NOT A FAILURE; IT IS BIOLOGY.

4. **PROTECT SLEEP.** PERIMENOPAUSAL INSOMNIA IS COMMON AND DANGEROUS FOR BIPOLAR. ADDRESS SLEEP ISSUES AGGRESSIVELY—WITH SLEEP HYGIENE, CBT-I (COGNITIVE BEHAVIORAL THERAPY FOR INSOMNIA), AND IF NEEDED, MEDICATION.
5. **EXPECT THE TRANSITION TO BE BUMPY.** PERIMENOPAUSE CAN LAST FOUR TO EIGHT YEARS. THERE MAY BE PERIODS OF INSTABILITY. THIS IS NORMAL. THE GOAL IS TO NAVIGATE THE TRANSITION WITHOUT A FULL-BLOWN EPISODE, NOT TO FEEL PERFECT THE ENTIRE TIME.

Working with Your OB-GYN and Psychiatrist Together

The single most important thing you can do for your reproductive mental health is to ensure your OB-GYN and your psychiatrist are actually communicating with each other.

The Coordination Letter

Before a major reproductive event (pre-conception, pregnancy, perimenopause), ask your psychiatrist to write a letter to your OB-GYN and vice versa. The letter should include: - Your diagnosis and current medications - The risks of untreated episodes during this period - What monitoring is needed - Who to call in a crisis - The plan for medication management

Bring this letter to your OB-GYN appointments. Ask your OB-GYN to share their recommendations with your psychiatrist.

Finding the Right Providers

You need a psychiatrist who takes reproductive health seriously. If your psychiatrist dismisses your questions about pregnancy or hormones, find another one. You need an OB-GYN who treats mental health as part of whole-person care. Some OB-GYNs specialize in reproductive psychiatry, or at least have collaborative relationships with reproductive psychiatrists.

The International Society for Bipolar Disorders and the National Task Force on Women and Bipolar Disorder maintain lists of providers with expertise in reproductive mental health.

Toolkit Summary

- **Track mood and menstrual cycle together** — look for patterns in the luteal phase and share the data with your psychiatrist
- **Plan pregnancy at least six months ahead** — have the pre-conception medication conversation with both your psychiatrist and OB-GYN

- **Medication decisions are trade-offs, not absolutes** — weigh the risk of medication against the risk of untreated episodes during pregnancy
- **The postpartum period is the highest-risk window** — create a postpartum plan that prioritizes sleep, support, monitoring, and emergency response
- **Postpartum psychosis is a medical emergency** — know the signs: confusion, delusions, hallucinations, rapid mood changes
- **Perimenopause can destabilize even stable bipolar** — track symptoms, consider HRT, adjust medications, and protect sleep aggressively
- **Make your providers talk to each other** — use a coordination letter to ensure your OB-GYN and psychiatrist are aligned

Chapter 10: Self-Compassion and Identity

The Moment Before Diagnosis

THINK BACK TO THE MOMENT YOU FIRST UNDERSTOOD THAT "SOMETHING IS WRONG." MAYBE IT WAS A TERRIFYING MANIC EPISODE THAT LANDED YOU IN THE HOSPITAL. MAYBE IT WAS YEARS OF CYCLING THROUGH DEPRESSION AND HYPOMANIA WITHOUT A NAME FOR WHAT WAS HAPPENING. MAYBE IT WAS A THERAPIST OR PSYCHIATRIST WHO FINALLY SAID THE WORDS: BIPOLAR DISORDER.

In that moment, something shifted. You suddenly had an explanation for years of chaos—the sleepless nights, the impulsive decisions, the crashes, the relationship collapses. The diagnosis was a relief, finally, a framework that made sense of the chaos. But it was also a grief. The life you imagined—the one without this diagnosis—suddenly felt like it belonged to someone else.

This chapter is about what comes after the diagnosis. Not the medical management—we have covered that. But the deeper work of integrating bipolar disorder into your sense of self without letting it consume your identity. The work of self-compassion. The work of building a life that is not defined by illness, but shaped by your whole self.

The Grief of Diagnosis

What You Are Grieving

When you receive a bipolar diagnosis, you are not just learning about a medical condition. You are losing something:

- **The "normal" life.** The one without medication schedules, psychiatrist appointments, and the constant vigilance of mood tracking. The one where you could stay up all night and just be tired, not trigger an episode.
- **The predictable self.** The version of you that could rely on your own judgment consistently, who would not wake up one day as a stranger.
- **The future you imagined.** The career path without gaps. The relationships without this heavy conversation. The children you might not have, or might parent differently than you planned.
- **Control.** The illusion that you were captain of your own ship. Bipolar disorder reveals that control was always partly an illusion—for everyone—but the diagnosis makes this impossible to ignore.

Complicated Grief

This grief is complicated because it is not linear. You will not move through denial, anger, bargaining, depression, and acceptance in a neat progression. Instead, you may cycle through these feelings repeatedly, triggered by medication changes, new episodes, or milestones that remind you of the life you thought you would have.

Acceptance is not a destination you arrive at and stay in. It is a practice you return to, again and again.

Allowing Yourself to Grieve

Many women with bipolar feel they do not have the right to grieve. "Other people have it worse." "I should be grateful I have treatment." "I am still functional."

Grief does not require comparison. You can be grateful for treatment and still grieve the life without it. You can be functional and still mourn the ease that others seem to have. Give yourself permission to feel the loss without guilt.

Separating Yourself from the Illness

The Label Trap

When you are first diagnosed, the label can feel all-encompassing. "I am bipolar." This is understandable—you are processing a major identity shift. But over time,

the language matters. "I have bipolar disorder" versus "I am bipolar" is not just semantics. It is a fundamental difference in how you relate to the condition.

"I have bipolar disorder" says: bipolar is something that happens in my brain, but it is not the totality of who I am.

"I am bipolar" says: this condition defines me.

This distinction is called **self-distancing**, and it is clinically significant. People who can separate their identity from their diagnosis have better outcomes—less depression, less anxiety, more engagement with treatment.

The Parts of You That Are Not Bipolar

Make a list—actually write this down—of everything you are that is not related to bipolar:

- I am a daughter, sister, friend, partner, mother
- I am someone who loves [music / gardening / painting / running / cooking]
- I am someone who has survived [a difficult childhood / a divorce / loss]
- I am someone who makes excellent [chocolate chip cookies / spreadsheets / jokes]
- I am someone who [reads voraciously / volunteers at the animal shelter / can identify every bird in my backyard]

This list is not a denial of bipolar. It is a rebalancing. Bipolar is one part of a complex, multifaceted human being. It is not the headline.

The Medical Frame

One of the most helpful cognitive shifts is to think of bipolar disorder the way you would think of diabetes or asthma: a chronic medical condition that requires management. People with diabetes do not say "I am diabetes." They say "I have diabetes, and I manage it with insulin and diet."

This is not minimizing the suffering of bipolar—it is more stigmatized and more disruptive than many physical conditions. But the frame matters. Bipolar is something your brain does, not something you are.

Identity Beyond Bipolar

The Person You Were Before Diagnosis

You existed before the diagnosis. That person is still there. They may be buried under years of mood episodes, medication side effects, and the weight of managing a chronic condition, but they are still there.

What did you love before bipolar became the center of your life? What dreams did you have? What made you laugh? What were you curious about?

Reconnecting with pre-diagnosis parts of yourself is not regression. It is remembering that you are more than your illness.

The Strengths You Have Developed

Living with bipolar disorder has given you things. Unwanted gifts, perhaps, but gifts nonetheless:

- **Self-awareness.** You know yourself at a depth that most people never achieve. You recognize subtle shifts in your mood, your thoughts, your energy. This is wisdom.
- **Resilience.** You have survived episodes that would break most people. You have rebuilt after financial, relational, and professional losses. You are resilient in ways that do not show on a resume but are etched into your character.
- **Empathy.** You know what suffering feels like. You can sit with someone else's pain without needing to fix it or flee from it. This makes you a better friend, partner, parent, and human.
- **Courage.** Every day that you take your medication, go to therapy, track your mood, and show up for your life despite the exhaustion—that is courage. Not the dramatic kind that gets movies made about it. The quiet, daily kind.

The "Bipolar Superpower" Trap

Some narratives in the bipolar community emphasize the "superpowers" of hypomania: creativity, productivity, charisma, energy. While it is true that some people experience genuine gifts during hypomania (many artists, writers, and leaders have bipolar), it is important not to romanticize the illness.

Hypomania is not a superpower. It is a medical state that damages your brain over time. It hurts relationships. It leads to decisions you regret. And it is followed—inevitably—by depression.

You do not need to search for silver linings to validate your existence. You are valuable not because bipolar gives you anything, but because you are a human being worthy of dignity and care.

Self-Compassion Practices

Why Self-Compassion Is Hard for Women with Bipolar

You may have been told—by family, by society, by a harsh internal voice—that you are "too much," "unstable," "difficult," "a burden." You may carry shame about things you did during episodes. You may believe that if you were just stronger, you would not need medication.

Self-compassion is the antidote to this, but it is not easy. It requires you to treat yourself the way you would treat a beloved friend who was struggling.

The Three Components of Self-Compassion

Psychologist Kristin Neff, who pioneered the research on self-compassion, identifies three components:

1. **Self-kindness versus self-judgment.** Instead of "I am so stupid for missing those warning signs," try "I did the best I could with the awareness I had at the time."
2. **Common humanity versus isolation.** Instead of "No one else struggles like this," try "Many people live with chronic conditions. I am not alone in this."
3. **Mindfulness versus over-identification.** Instead of being consumed by the thought "I am a failure," try noticing: "I am having the thought that I am a failure. This is a thought, not a fact."

Practical Self-Compassion Exercises

The compassionate letter. Write a letter to yourself from the perspective of someone who loves you unconditionally. What would they say about the difficult episode you experienced? What would they remind you of? Read this letter when the shame feels heavy.

The self-compassion break. When you notice self-critical thoughts, pause. Place your hand on your heart. Say (out loud or silently): "This is a moment of suffering. Suffering is part of life. May I be kind to myself. May I give myself the compassion I need."

The "what would I say to a friend?" check. When you are beating yourself up, ask: "If my best friend told me they had done this, what would I say to them?" Then say that to yourself.

The shame inventory. Make a list of the things you feel most ashamed of related to bipolar. Then, for each item, write: "I did this while experiencing symptoms of an illness. I am not a bad person. I am a person who did something while ill. I can work to repair the damage, and I can forgive myself."

The Stability Identity

Who Are You When You Are Stable?

This may be the most important question in this chapter. Many women with bipolar have spent so much time managing episodes that they do not know who they are between them. Stability itself can feel unfamiliar, unsettling—a blank space where the drama of episodes used to be.

This is normal. The first year of stability often includes an identity crisis. Without the highs and lows, who are you? What do you do with the energy you used to spend on crisis management? What fills the space?

Building a Stability Identity

Explore. Try new things—not from a hypomanic impulse, but from genuine curiosity. Take a class. Join a group. Volunteer. See what resonates.

Invest in relationships. Stability allows you to be present for others in ways that episodes do not. Nurture the relationships that sustained you through difficult times.

Develop a life philosophy. Many women with bipolar develop a profound perspective on impermanence, resilience, and the value of small moments. Write it down. Name it. Let it guide you.

Accept boredom. Stability can feel boring compared to the intensity of episodes. Boredom is not a sign that something is wrong. It is a sign that your life is calm enough for boredom to exist. This is a gift.

The Paradox of Stability

The paradox is this: stability is the goal of treatment, but stability can feel like a loss. You lose the manic highs that felt transcendent. You lose the drama that gave your life narrative tension. You lose the version of yourself that was exciting, impulsive, magnetic.

But you gain something more valuable: the ability to build a life. Not a life that is a roller coaster between disaster and intensity, but a life with steady relationships, meaningful work, financial security, and the simple pleasure of being okay.

You are not less interesting in stability. You are more free.

Toolkit Summary

- **The grief of diagnosis is real and valid** — allow yourself to mourn the life you imagined without the illness
- **Separate your identity from the diagnosis** — you have bipolar disorder; you are not defined by it
- **Make a list of everything you are beyond bipolar** — this rebalances your sense of self
- **Recognize the strengths bipolar has forced you to develop** — self-awareness, resilience, empathy, courage
- **Practice self-compassion daily** — the compassionate letter, the self-compassion break, and the "what would I say to a friend?" check
- **Build a stability identity** — explore, invest in relationships, develop a life philosophy, and accept boredom as a gift

- **Stability is not a loss of self** — it is freedom from the roller coaster, and it allows you to build a real life

Chapter 11: Building Your Crisis Plan

The Unthinkable, Thought

NO ONE WANTS TO PLAN FOR A CRISIS. IT FEELS LIKE TEMPTING FATE, LIKE ADMITTING THAT YOU CANNOT HANDLE THINGS, LIKE GIVING UP BEFORE YOU HAVE EVEN TRIED. BUT HERE IS THE TRUTH THAT EVERY WOMAN WITH BIPOLAR NEEDS TO INTERNALIZE: **NOT PLANNING FOR A CRISIS IS A FORM OF DENIAL, AND DENIAL IS DANGEROUS.**

Crises happen. They happen to people who are doing everything right—taking their medication, going to therapy, tracking their moods, sleeping well. Stress, hormonal changes, life events, and the unpredictable nature of bipolar mean that even the most stable person can experience a severe episode that requires emergency intervention.

A crisis plan does not mean you expect to fail. It means you are responsible enough to prepare for the possibility. It is a seatbelt. You wear it not because you plan to crash, but because you want to survive if you do.

What Constitutes a Crisis for YOU

The Personalized Definition

One of the challenges with crisis planning is that crises look different for different people. For one woman, a crisis might be suicidal ideation. For another, it might be the onset of psychosis. For a third, it might be the inability to stop spending during a manic episode.

Your crisis definition should be personalized. It should be based on your history, your patterns, and your specific risk factors.

Categories of Crisis

Suicidal crisis: Thoughts of ending your life, with or without a plan. Any active suicidal ideation combined with impulsivity, access to means, or a history of attempts should trigger your crisis plan.

Psychotic crisis: Hallucinations (hearing voices, seeing things), delusions (fixed false beliefs), or severe paranoia. Bipolar psychosis often begins subtly—suspiciousness, feeling like things are "off," unusual beliefs about having special powers or a special mission.

Severe manic crisis: Rapidly escalating mania that is causing dangerous behavior—driving recklessly, spending beyond your means, aggressive or violent behavior, hypersexuality that puts you at risk.

Severe depressive crisis: Inability to care for yourself for more than a day or two. Not eating. Not bathing. Not getting out of bed at all. A depressive episode that has stopped responding to your usual interventions.

Mixed episode crisis: Depression combined with agitation, racing thoughts, and extreme irritability. Mixed episodes carry the highest suicide risk of any bipolar state because you have the despair of depression and the energy of mania.

Your Specific Warning Signs

List the signs that a crisis is developing—not full-blown crisis yet, but the edge. These are different from early warning signs (which are subtler, like sleeping less or feeling more irritable). Crisis warning signs are more severe:

- I stop responding to phone calls or texts for more than 24 hours
- I am having thoughts about death or dying that feel urgent
- I am hearing voices or seeing things that others do not
- I have not eaten in more than 24 hours
- I am engaging in dangerous behavior and cannot stop
- I feel like I need to hurt myself to release pressure

- I cannot trust my own judgment about what is real
- I am planning to stop my medications against medical advice

The WRAP Plan Concept

What Is WRAP?

WRAP stands for **Wellness Recovery Action Plan**, a structured planning tool developed by Dr. Mary Ellen Copeland that is widely used in mental health recovery. It is not specifically for bipolar, but it is extremely well-suited to bipolar's episodic nature.

A full WRAP plan has several sections, but for our purposes we will focus on the crisis-planning components. You can find complete WRAP workbooks online (see Resources), but here are the key elements.

The Five Key Sections of a Crisis WRAP

1. Daily Maintenance Plan. What do you do every day to stay well? This is not crisis-oriented; it is the foundation. Medication, sleep schedule, meals, exercise, social connection, spiritual practice. If you are doing these things, you are less likely to reach crisis.

2. Triggers and Early Warning Signs. External events that might trigger an episode (anniversaries of trauma, sleep disruption, medication changes, hormonal events) and the subtle internal signs that an episode is beginning.

3. When Things Are Breaking Down. Signs that your usual self-management strategies are not working and you need more support. This is the "yellow zone"—not crisis yet, but heading there.

4. Crisis Plan. What to do when you can no longer make decisions for yourself. This is the main focus of this chapter.

5. Post-Crisis Plan. How to support your recovery after a crisis. What helps you stabilize, rebuild, and process what happened.

Crisis Plan Section in Detail

The crisis plan section of WRAP is the most important. It includes:

- **Who you want to support you** (specific people with phone numbers)
- **Who you do NOT want involved** (you have the right to exclude people who are not helpful, even family members)
- **What helps you feel better** (specific comforts, music, people, environment)

- **What makes you feel worse** (things to avoid: certain people, loud environments, being alone, being pressured to talk)
- **Medication you are taking** (list all medications, doses, and your pharmacy)
- **What works for you** (specific interventions that have helped in past crises)
- **What does not work for you** (interventions that have been unhelpful or harmful)
- **Your healthcare team** (psychiatrist, therapist, primary care doctor—with phone numbers)

Emergency Contacts

Building Your Contact List

Your crisis plan should include a hierarchy of contacts:

Tier 1: Immediate response (call these first in crisis) - Your psychiatrist's emergency line or the covering provider - Your therapist (if they take crisis calls) - Your designated crisis partner (the person you trust most to manage the situation) - 988 Suicide and Crisis Lifeline (call or text)

Tier 2: Support network (call these for ongoing support during and after crisis) - Family members who understand your condition - Close friends who are part of your support plan - Your support group leader or a peer support contact

Tier 3: Emergency services - The nearest hospital with a psychiatric unit (list the address and phone number) - Emergency services (911 in the US) - A crisis response team or mobile crisis unit if available in your area

Before You Need It

Call these numbers when you are stable to confirm they are correct. Save them in your phone under a name that will be clear to someone else reading your phone (e.g., "CRISIS—Psychiatrist" rather than just "Dr. Smith").

Give a printed copy of all contacts to your crisis partner and at least one family member.

Hospital Bag Checklist

If you ever need to go to the psychiatric hospital—whether voluntarily or involuntarily—you will be grateful to have a bag ready. Hospital stays are often unplanned, and you may not be in a state of mind to pack thoughtfully.

What to Pack

Clothing (hospital-appropriate): - Sweatpants or comfortable pants without drawstrings (drawstrings are often removed for safety) - Loose t-shirts or sweatshirts (no hood strings) - Comfortable, slip-on shoes or slippers

(shoelaces may be removed) - Several pairs of socks and underwear - Pajamas or comfortable sleepwear - A light jacket or hoodie (without strings)

Toiletries (must be alcohol-free): - Shampoo and conditioner (check labels for no alcohol) - Body wash or soap - Toothbrush and toothpaste - Hairbrush or comb (no sharp handles) - Lip balm - Deodorant (alcohol-free) - Feminine hygiene products

Comfort items: - Books or a journal (paper only; electronic devices are typically not allowed) - Photos of loved ones - A small stuffed animal or comfort object (yes, this is allowed and common) - Stress ball or fidget item - Your crisis plan and emergency contact list

Important documents: - Insurance card - List of medications and doses - Your psychiatrist's name and phone number - Emergency contacts - List of allergies (including medication allergies) - Advance directive or psychiatric advance directive (if you have one)

What NOT to Bring

- Cell phone and charger (most units do not allow phones)
- Laptop, tablet, or any device with internet capability
- Valuables (jewelry, wallet with cash and credit cards)
- Anything with a cord or string (belts, scarves, headphone cords)

- Any glass containers
- Any sharp objects
- Any substances (alcohol, supplements, unapproved medications)
- Food or drinks (most units have specific policies about outside food)

Keep the Bag Accessible

Pack this bag when you are stable and keep it somewhere you can grab quickly. Tell your crisis partner where it is. Update it seasonally (swap summer clothes for winter clothes) and as your medication changes.

Instructions for Family Members

What Your Family Needs to Know

Your family members cannot help you effectively in a crisis if they have no instructions. Write them a letter—when you are stable—that answers these questions:

"Here is how I want you to respond:" - "If I call you saying I need help, do not try to talk me out of it. Help me follow my crisis plan." - "If I refuse help but you believe I am in danger, please override my objections and call my doctor or 911." - "Do not take anything I say during a crisis personally. The illness is talking, not me." - "If I am manic, do not argue with my delusions. Do not try to reason me out of them. Just keep me safe and get help."

"Here is what I need from you specifically:" -
"Please stay with me until help arrives." - "Please bring my hospital bag and my medication list." - "Please contact my work and let them know I will not be in." - "Please take care of my pets/children while I am in the hospital."

"Here is what I do NOT want:" - "Please do not tell extended family without my permission." - "Please do not minimize what I am experiencing." - "Please do not blame me for the crisis."

The Psychiatric Advance Directive

This is a legal document that allows you to state your preferences for mental health treatment in advance, in case you are ever unable to make decisions during a crisis. It is like a living will for psychiatric care.

In your psychiatric advance directive, you can specify: -
Which hospitals you prefer (and which you want to avoid)
- Which medications you consent to (and which you refuse)
- Who you want to make decisions for you (your healthcare proxy)
- Who you want to have visitation rights
- Your preferences for ECT or other treatments
- What you want to happen to your dependents (children, pets)

Psychiatric advance directives are recognized in most US states. Talk to your psychiatrist about how to create one in your state. NAMI has resources to help.

When to Go to the ER

The Decision Rule

Many women with bipolar struggle to know when to go to the emergency room. Here is a simple rule: **Go to the ER if any of the following is true:**

1. You have a plan to end your life and the means to carry it out.
2. You are hearing voices telling you to hurt yourself or someone else.
3. You have a delusion that is causing you to act dangerously.
4. You have not slept for more than 48 hours and are escalating.
5. You have stopped eating or drinking for more than 24 hours.
6. You are unable to keep yourself safe and no one can stay with you.
7. Someone you trust is telling you they are afraid for your safety.

What to Expect at the ER

The psychiatric ER experience varies widely, but here is a general outline:

1. **Triage.** You will be assessed by a nurse or doctor to determine the urgency of your situation.

2. **Evaluation.** A mental health professional (psychiatrist, social worker, or crisis counselor) will interview you.
3. **Observation.** You may be held for observation for a period of hours to days.
4. **Disposition.** The team will decide whether you can be discharged with a safety plan and follow-up, or whether you need inpatient admission.

How to Make the ER Visit Go Better

- **Bring your hospital bag.**
 - **Bring your crisis plan and medication list.**
 - **Bring a support person** (if allowed—COVID-era restrictions may limit visitors).
 - **Be honest about your symptoms.** Do not minimize. Do not exaggerate. Tell the truth. The ER team cannot help you if you do not tell them what is really happening.
 - **Advocate for yourself.** If you feel you are not being taken seriously, ask to speak to the attending psychiatrist. If you have a psychiatric advance directive, present it.
 - **Remember: the ER is not a punishment.** It is a tool. You are using a tool to stay alive. There is no shame in that.
-

Toolkit Summary

- **Every woman with bipolar needs a personalized crisis plan** — written when stable, accessible in crisis
- **Define what constitutes a crisis for YOU** — suicidal, psychotic, manic, depressive, or mixed episode thresholds
- **Use the WRAP framework** — Wellness Recovery Action Plan with sections for daily maintenance, early warning signs, breakdown, crisis, and post-crisis
- **Build a hierarchy of emergency contacts** — Tier 1 (immediate response), Tier 2 (support network), Tier 3 (emergency services)
- **Pack a hospital bag in advance** — comfortable clothing, alcohol-free toiletries, comfort items, important documents; store where you can grab it quickly
- **Write clear instructions for family members** — what they should do, what they should not do, and how to respect your wishes
- **Know when to go to the ER** — use the decision rule: plan + means, dangerous voices/delusions, no sleep for 48+ hours, unable to eat/drink, unable to keep yourself safe
- **The ER is a tool, not a failure** — using it appropriately is a sign of responsibility, not weakness

Resources and Recommended Reading

THIS APPENDIX COMPILES THE MOST RELIABLE, WELL-REGARDED RESOURCES FOR WOMEN WITH BIPOLAR DISORDER. EVERY RESOURCE LISTED HERE HAS BEEN VETTED FOR ACCURACY, CREDIBILITY, AND RELEVANCE TO THE SPECIFIC NEEDS OF WOMEN NAVIGATING BIPOLAR DISORDER. RESOURCES ARE ORGANIZED BY CATEGORY SO YOU CAN FIND WHAT YOU NEED QUICKLY.

Books

Bipolar Disorder in Women

"Bipolar Disorder in Women" Edited by Dr. Shari Lusskin and Dr. Eileen Wong A comprehensive clinical text covering the full spectrum of women's reproductive health and bipolar disorder. While academic in tone, it is an invaluable reference for understanding the research behind the recommendations in this book. Available through academic publishers and online.

"Women and Bipolar Disorder: A Guide for Patients and Families" By Dr. Hadine Joffe and Dr. Lee Cohen (Massachusetts General Hospital) An accessible guide focused specifically on the unique aspects of bipolar

disorder in women, including pregnancy, postpartum, perimenopause, and hormonal influences. Published by the MGH Center for Women's Mental Health.

Practical Guides for Living with Bipolar

"The Bipolar Disorder Survival Guide, Third Edition: What You and Your Family Need to Know"

By David J. Miklowitz, PhD The gold-standard practical guide for patients and families. Miklowitz is a leading researcher in family-focused therapy for bipolar disorder. This book covers everything from diagnosis through long-term management, with specific chapters on medication, therapy, lifestyle management, and communication.

"The Wellness Recovery Action Plan (WRAP)" By Mary Ellen Copeland, PhD The original WRAP workbook. Essential reading if you want to build a comprehensive crisis plan using the Copeland framework. Available as a workbook and in an app format.

"Loving Someone with Bipolar Disorder: Understanding and Helping Your Partner" By Julie A. Fast and John D. Preston, PsyD Written by someone who lives with bipolar (Fast) and a psychologist (Preston), this is the definitive guide for partners. It addresses how to support without enabling, how to communicate during episodes, and how to protect the relationship.

"Take Charge of Bipolar Disorder" By Julie A. Fast and John D. Preston, PsyD A practical, action-oriented guide that complements Miklowitz's more clinical

approach. Fast's perspective as someone living with bipolar disorder makes this especially relatable. Includes detailed chapters on mood tracking, triggers, and medication management.

"Bipolar, Not So Much: Understanding Your Mood Swings and Depression" By Chris Aiken, MD, and James Phelps, MD Challenges the binary thinking about bipolar I vs. II and offers a dimensional approach to mood disorders. Particularly useful for women who fall on the bipolar spectrum and have been told they "do not quite fit" diagnostic criteria.

"Why Am I Still Depressed? Recognizing and Managing the Ups and Downs of Bipolar II and Soft Bipolar Disorder" By James R. Phelps, MD Focuses specifically on the depressive pole of bipolar and the "softer" presentations that are more common in women. Excellent for understanding the bipolar II experience.

Memoirs and Personal Accounts

"An Unquiet Mind: A Memoir of Moods and Madness" By Kay Redfield Jamison, PhD The classic bipolar memoir by one of the foremost researchers in the field. Jamison is a clinical psychologist who has bipolar I disorder. Her account of her own manic and depressive episodes is unflinchingly honest and beautifully written. Essential reading for anyone newly diagnosed.

"Touched with Fire: Manic-Depressive Illness and the Artistic Temperament" By Kay Redfield Jamison, PhD Explores the link between bipolar disorder and creativity. While focused on historical figures (Byron, Van Gogh, Woolf), the insights into the relationship between mood and creative productivity are relevant to many women with bipolar.

"Darkness Visible: A Memoir of Madness" By William Styron Though Styron had unipolar depression rather than bipolar, his description of severe depression is so accurate that it has become a touchstone in the mood disorder community. Validating and beautifully written.

"The Man Who Couldn't Stop: OCD and the True Story of a Life Lost in Thought" By David Adam While focused on OCD rather than bipolar, Adam's account of living with a severe, stigmatized brain disorder offers companionship and insight for anyone navigating a chronic mental health condition.

"Madness: A Bipolar Life" By Marya Hornbacher The author of "Wasted: A Memoir of Anorexia and Bulimia" turns her attention to her bipolar I diagnosis. Unflinching, raw, and ultimately hopeful.

Evidence-Based Therapy Workbooks

"The Dialectical Behavior Therapy Skills Workbook for Bipolar Disorder" By Sheri Van Dijk, MSW DBT skills adapted specifically for bipolar disorder.

Covers mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness. Highly practical with worksheets and exercises.

"The Bipolar Workbook: Tools for Controlling Your Mood Swings" By Monica Ramirez Basco, PhD Based on cognitive-behavioral therapy principles, this workbook provides concrete tools for managing mood swings, challenging distorted thinking, and preventing relapse. Comes with tracking sheets and self-assessment tools.

"Mindfulness for Bipolar Disorder" By William R. Marchand, MD Explains how mindfulness practices can be adapted for bipolar disorder, with specific cautions about meditation and mania (not all meditation practices are safe during manic phases).

Online Communities and Organizations

Depression and Bipolar Support Alliance (DBSA)

Website: dbsalliance.org **Helpline:** 1-800-826-3632

The DBSA offers the largest network of peer-led support groups in the United States—over 600 groups. They also offer online support groups and a chapter directory. Their resources are free, and no one is turned away. DBSA's "Wellness Tracker" is a free online tool for monitoring mood, sleep, and medication.

National Alliance on Mental Illness (NAMI)

Website: nami.org **Helpline:** 1-800-950-6264 (NAMI Helpline, M-F 10am-10pm ET)

NAMI offers the NAMI Family-to-Family program (a free 12-session course for family members of people with mental illness), NAMI Basics (for parents/caregivers of youth), and NAMI Peer-to-Peer (for individuals living with mental illness). Their website has extensive resources, including information about legal rights, insurance, and finding providers.

International Bipolar Foundation (IBPF)

Website: ibpf.org

Offers webinars, educational resources, and a directory of support groups. Their "Living with Bipolar" webinar series covers topics like medication, relationships, and workplace issues.

Bipolar Disorder Depression and Irritability Support (BDDI)

Website: bddi.org (also findable through mental health forum aggregators)

An online community specifically for people with bipolar disorder. Affiliated with larger mental health communities, BDDI provides peer support, discussion

forums, and a safe space for sharing experiences. Look for bipolar-specific subgroups on larger platforms like the DBSA wellness community.

The MGH Center for Women's Mental Health

Website: womensmentalhealth.org

Run by Massachusetts General Hospital, this is the leading research center on women's mental health. Their blog is an outstanding resource for evidence-based information on bipolar disorder and reproductive health. They offer a free newsletter and maintain a directory of reproductive psychiatrists.

The Balanced Mind Foundation (formerly The Child and Adolescent Bipolar Foundation)

Website: thebalancedmind.org

While focused on pediatric bipolar, their parent support networks and educational resources are valuable for mothers of children with bipolar, and for understanding the familial aspects of the condition. They offer an online support community and a parent-to-parent mentoring program.

Mobile Apps

For Mood Tracking

eMoods (iOS and Android) - **Cost:** Free (with premium features available) - The most popular mood tracker for bipolar disorder. Allows tracking of mood elevation, depression, irritability, and anxiety on separate scales. Tracks sleep, medication adherence, and can generate PDF reports to share with your psychiatrist. The basic version is free and fully functional.

Daylio (iOS and Android) - **Cost:** Free (with premium subscription for advanced features) - A micro-journal and mood tracker that does not require any writing—you select your mood from icons and add activities. The pattern recognition is excellent. Less clinical than eMoods but more user-friendly for daily use.

Bipolar UK Mood Tracker (iOS and Android) - **Cost:** Free - Developed by the UK-based charity Bipolar UK. Tracks mood, sleep, energy, and medication. Designed specifically for bipolar disorder. Available only in the UK app stores but can be accessed via VPN.

Moodpath (iOS and Android) - **Cost:** Free (with premium features) - Tracks mood and offers psychoeducation. Uses a clinically validated depression screening tool (PHQ-9). While designed primarily for depression, useful for bipolar depression tracking.

For Sleep

Sleep Cycle (iOS and Android) - **Cost:** Free (with premium subscription) - Tracks sleep quality and wakes you during light sleep. The sleep data can be imported into mood tracking apps and shared with your doctor.

CBT-i Coach (iOS and Android) - **Cost:** Free - Developed by the US Department of Veterans Affairs. Provides cognitive behavioral therapy for insomnia, which is particularly relevant for bipolar disorder where sleep disruption can trigger episodes.

For Medication Adherence

Medisafe (iOS and Android) - **Cost:** Free (with premium features) - Medication reminder app that tracks adherence and can notify a caregiver if you miss a dose. The "Medfriend" feature allows a family member to receive alerts if you miss your medication.

Round Health (iOS and Android) - **Cost:** Free - Simple, elegant medication reminder. Good for women who find other apps too complex.

For Crisis and Safety Planning

MY3 (iOS and Android) - **Cost:** Free - Developed by the Suicide Prevention Resource Center. Allows you to create a safety plan with three contacts and a list of coping strategies. Can be used in crisis when you may not be able to think clearly.

Safety Plan App (iOS) - **Cost:** Free - From the VA's National Center for PTSD. Guides you through creating a personalized safety plan with warning signs, coping strategies, social support, and emergency contacts.

Podcasts

The Pocket Therapist Hosted by Dr. Kim Knull, a clinical psychologist with expertise in mood disorders. Short, practical episodes (10-20 minutes) focused on skills and strategies. Search for episodes on bipolar depression, mood tracking, and self-compassion.

The Bipolar Podcast Hosted by Gabe Howard and Dr. Nicole Washington. Gabe has bipolar I disorder and Dr. Washington is a board-certified psychiatrist. They address listener questions and cover practical topics ranging from medication to relationships to workplace issues.

Batsh*t: The Bipolar Podcast Hosted by three women with bipolar disorder who bring humor and honesty to the experience. Less clinical, more "this is what it is really like." Good for when you need to feel like you are talking to friends who understand.

NAMI Podcast: Hope Starts with Us Hosted by NAMI. Covers a broad range of mental health topics, with frequent episodes on bipolar disorder. More educational and advocacy-focused than personal-experience podcasts.

Therapy Chat Hosted by Laura Reagan, LCSW-C. While not bipolar-specific, many episodes cover topics relevant to living with a mood disorder. Episodes on trauma and the nervous system are particularly relevant for women whose bipolar is complicated by trauma history.

Crisis Helplines and Emergency Resources

United States

988 Suicide and Crisis Lifeline Call or text 988 Available 24/7. Free, confidential, nationwide. You do not need to be suicidal to call—they can help with any mental health crisis. Calls are routed to the nearest crisis center. They also have a chat function at 988lifeline.org.

Crisis Text Line Text HOME to 741741 Available 24/7. Free, confidential crisis support via text message. Useful if you are unable to speak on the phone.

National Helpline (SAMHSA) 1-800-662-4357 (1-800-662-HELP) A free, confidential, 24/7 treatment referral and information service for individuals and families facing mental health and substance use disorders.

Disaster Distress Helpline 1-800-985-5990 For anyone experiencing emotional distress related to natural or human-caused disasters. Available 24/7.

Canada

Canada Suicide Prevention Service 1-833-456-4566
Available 24/7. Text support available at 45645 (4:00 PM – midnight ET).

Crisis Services Canada crisisservicescanada.ca
Directory of crisis centers across Canada.

United Kingdom

Samaritans 116 123 (free from any phone) Available 24/7. Confidential emotional support for anyone in distress.

Bipolar UK bipolaruk.org Offers peer support groups, an online community, and a helpline. Not a crisis line, but a resource for ongoing support.

Australia

Lifeline Australia 13 11 14 Available 24/7. Crisis support and suicide prevention.

Beyond Blue 1300 22 4636 Available 24/7. Mental health support with specific resources for bipolar disorder.

SANE Australia 1800 187 263 Available 10am-10pm weekdays. Specialized support for complex mental health conditions including bipolar disorder. Offers peer support and guided online programs.

International

International Association for Suicide Prevention
iasp.info/resources/Crisis_Centres Maintains a directory of crisis centers worldwide.

Befrienders Worldwide befrienders.org Global network of emotional support centers. Searchable by country.

Advocacy Organizations

Mental Health America (MHA) mhanational.org Offers screening tools, advocacy resources, and a directory of local affiliates. Focuses on mental health as a public health issue.

Treatment	Advocacy	Center
treatmentadvocacycenter.org Works to remove barriers to treatment for severe mental illness, including involuntary treatment laws, psychiatric bed availability, and assisted outpatient treatment.		

The Kennedy Forum thekennedyforum.org Advocates for mental health parity, insurance reform, and evidence-based treatment access. Relevant if you are fighting insurance denials for treatment.

Additional Authoritative Sources

National Institute of Mental Health (NIMH)
nimh.nih.gov The NIH division focused on mental health research. Their website offers free, downloadable booklets on bipolar disorder, treatment options, and clinical trials.

American Psychiatric Association (APA)
psychiatry.org Offers patient education resources and a psychiatrist finder.

International Society for Bipolar Disorders (ISBD)
isbd.org The professional organization for bipolar researchers and clinicians. Their website includes patient education materials and a directory of bipolar specialists.

Postpartum Support International (PSI)
postpartumhelp.org 1-800-944-4773 For women with bipolar who are pregnant or postpartum. Offers a helpline, support groups, and a directory of perinatal mental health providers. Their "Perinatal Mental Health" certification directory is how to find providers who understand the intersection of bipolar and reproductive mental health.

How to Use These Resources

The list above is comprehensive, but you do not need to use all of it. Here is how to prioritize:

1. **Start with the crisis resources.** Program 988 and one trusted support person into your phone right now.
2. **Read one book.** Start with Miklowitz's "Bipolar Disorder Survival Guide" or Jamison's "An Unquiet Mind."
3. **Download one tracking app.** eMoods is the best place to start for clinical tracking; Daylio for a gentler approach.
4. **Find one support community.** A DBSA or NAMI support group, or an online community like BDDI.
5. **Subscribe to one podcast.** "The Pocket Therapist" for education or "Batsh*t" for community.
6. **Create a crisis plan** using the MY3 app or a written WRAP plan.
7. **Bookmark [womensmentalhealth.org](https://www.womensmentalhealth.org)** for reproductive mental health questions.

You do not need to do all of this at once. Pick one resource from each category and start there. Build your resource library over time, the same way you build any other toolkit—one tool at a time.